ENGLISH FOR WORK / ENGLISH FOR THE DENTAL TEAM

Dental Assisting English

Clarify the request. Support the patient.



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Eight practical lessons. Eleven extended conversations.
Vocabulary, phrase banks, guided speaking, and explained answers.

Revised 2026 edition / Upper-intermediate to advanced

Your route through the book

01 Clarifying the appointment purpose, page 4

Reconcile a patient expectation with the schedule and arrange an explanation before assuming a treatment decision.

02 Room readiness and safe clarification, page 16

Raise an unresolved instrument-readiness check clearly and seek a verified replacement without waiving the practice process.

03 Chairside requests and read-backs, page 28

Identify the word you missed, request the distinguishing detail, and confirm the full chairside instruction before acting.

04 Imaging questions and authorization, page 38

Acknowledge a question about dental imaging and arrange clinical explanation without interpreting findings or promising treatment.

05 Patient comfort, questions, and pauses, page 51

Acknowledge dental anxiety, communicate a comfort concern, and request an agreed pause signal without guaranteeing sensations.

06 Records, identifiers, and factual handoffs, page 62

Report an identifier mismatch before using a record and hand off the facts without guessing which source is correct.

07 Laboratory cases and delivery expectations, page 75

Distinguish estimated delivery from clinical readiness and agree a useful laboratory update before confirming the patient plan.

08 Aftercare questions and follow-up calls, page 86

Capture a postoperative question accurately, route it to the clinician, and distinguish routine clarification from urgent concerns.

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Suggested study range: B2-C1. This is an editorial study range, not a certified proficiency assessment.

Clear language supports careful teamwork.

Dental assisting combines technical vocabulary, short chairside exchanges, and patient conversations that need time and empathy. A clear question can prevent a wrong assumption. A helpful explanation can acknowledge uncertainty without sounding dismissive.

Separate the appointment from the treatment

A schedule entry, a patient expectation, and a clinical treatment decision are different pieces of information. Explain the recorded visit and arrange clarification before assuming agreement to a procedure.
"The schedule shows a consultation; the dentist needs to clarify the plan with you."

Make a concern specific

Name the item, missing check, and action needed. Keep an unverified status distinct from a confirmed failure, but do not let schedule pressure turn missing evidence into permission to proceed.
"I cannot confirm the readiness record for this pack; we need the required check."

Check the word you missed

Chairside speech can be brief and interrupted. Ask for the distinguishing word and repeat the full request. Knowing two possible instruments does not tell you which one was requested.
"Did you mean the instrument tray or the impression tray?"

Route clinical questions without abandoning them

An assistant can recognize and communicate a question while staying within the actual role. Preserve the patient concern and identify the clinician or approved process that can answer it.
"I cannot interpret the image, but I will ask the dentist to explain what it shows."

Professional boundaries

All patients, staff, records, schedules, and cases are fictional. This book teaches workplace English, not dental treatment, radiographic interpretation, infection-control certification, or a universal scope of practice. Follow current local law, authorized duties, training, manufacturer instructions, and practice procedures. Use the appropriate urgent or emergency route for urgent concerns; routine callback examples do not replace it. United States and UK sources provide context, not interchangeable legal rules.

A Read the situation

LESSON 01 / CLARIFYING THE APPOINTMENT PURPOSE

Reconcile a patient expectation with the schedule and arrange an explanation before assuming a treatment decision.

> **A consultation is not a confirmed filling**
>
> Patient Lena arrives expecting a filling today. Assistant Amir sees a thirty-minute consultation on the schedule; no procedure is confirmed. The dentist is available to clarify the purpose before Lena decides whether to continue. Amir should acknowledge the time she has set aside, explain what the record actually shows, and preserve her questions about options, costs, and timing. He cannot promise a filling today, diagnose the tooth, or imply that attending a consultation means agreeing to whatever treatment might later be recommended.

People: Lena, Patient; Amir, Dental assistant.

Read the record

Choose one answer per question using the facts above.

1. What does the schedule show?

A. A thirty-minute consultation
B. A completed filling
C. A guaranteed procedure today
D. An emergency extraction

2. What does Lena expect?

A. A laboratory delivery next week
B. Only a telephone call
C. No discussion with a dentist
D. A filling today

3. Who can clarify the clinical plan?

A. The schedule automatically
B. The dentist before Lena decides whether to continue
C. Amir by guessing from the booking length
D. Another patient in the waiting room

Answers and explanations, page 115

B Vocabulary in use

CORE TERMS

consultation
An appointment for discussion and assessment, not automatic agreement to treatment.

attend a consultation

filling
Material placed to restore a prepared tooth area as clinically indicated.

schedule a filling

chief concern
The main issue the patient wants addressed.

clarify the chief concern

clinical examination
Assessment performed by the appropriate clinician.

arrange a clinical examination

treatment option
A possible approach requiring appropriate explanation and assessment.

compare treatment options

provisional recommendation
A recommendation not yet finalized or confirmed.

qualify a provisional recommendation

restoration
Repair or replacement of damaged tooth structure using an appropriate dental method.

discuss a restoration

appointment type
The category of visit recorded in the schedule.

verify the appointment type

presenting complaint
The problem or symptom reported at the visit.

record the presenting complaint

treatment plan
The proposed sequence of dental care discussed with the patient.

explain the treatment plan

informed consent
Agreement through the applicable process after relevant understandable information.

support informed consent

definitive treatment
Treatment intended to address the identified condition rather than an interim measure.

discuss definitive treatment

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

consultation CONSULTATION

attend a consultation

Keep the meaning precise: An appointment for discussion and assessment, not automatic agreement to treatment.

presenting complaint PRESENTING COMPLAINT

record the presenting complaint

Keep the meaning precise: The problem or symptom reported at the visit.

B Precision and professional shorthand

EXTEND YOUR RANGE

caries

Tooth decay; identification and management require clinical assessment.

discuss diagnosed caries

procedure code

A coded description of a dental service within a particular system.

verify the procedure code

coverage estimate

An estimate of relevant payment coverage, not a guarantee.

qualify a coverage estimate

review appointment

A follow-up visit to reassess an issue or treatment.

arrange a review appointment

clinical decision

A decision requiring the appropriate clinical assessment and authority.

refer a clinical decision

patient expectation

What the patient believes will happen.

acknowledge a patient expectation

restorative dentistry

Dental care concerned with repairing or restoring teeth and function.

explain restorative dentistry

treatment estimate

An estimated cost for specified proposed care.

request a treatment estimate

referral

A request for assessment or care by another appropriate provider.

confirm the referral

booking discrepancy

A difference between the recorded booking and the expected visit.

resolve a booking discrepancy

appointment duration

The allocated length of the visit.

confirm appointment duration

decision time

Time for the person to receive information and make the relevant choice.

allow decision time

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

definitive treatment DEFINITIVE TREATMENT

discuss definitive treatment

Keep the meaning precise: Treatment intended to address the identified condition rather than an interim measure.

coverage estimate COVERAGE ESTIMATE

qualify a coverage estimate

Keep the meaning precise: An estimate of relevant payment coverage, not a guarantee.

C Phrases that move the work forward

Acknowledge the expectation

I understand you expected a filling today.

Name the difference

That differs from the treatment you were expecting.

Invite the concern

What would you like the dentist to address first?

Keep the procedure unconfirmed

No filling is confirmed in the booking I can see.

Avoid a diagnosis

I cannot determine the treatment from the schedule.

Route cost questions

I will include your question about the treatment estimate.

Clarify timing

The allocated thirty minutes does not guarantee a procedure today.

Preserve the concern

I will tell the dentist that you expected treatment today.

State the record

The schedule shows a thirty-minute consultation.

Avoid blame

Let us clarify the appointment before making assumptions.

Explain the next step

The dentist can discuss the purpose and options with you.

Respect the decision

You can ask questions before deciding whether to continue.

Separate attendance and consent

Coming to this visit is not agreement to every possible procedure.

Qualify coverage

Any coverage estimate needs confirmation through the appropriate process.

Check understanding

Are we clear that the current booking is for consultation?

Close the handoff

We will clarify the plan and your questions before treating a procedure as agreed.

Precision in real exchanges

Purpose: Avoid blame
Useful wording: Let us clarify the appointment before making assumptions.

Purpose: Invite the concern
Useful wording: What would you like the dentist to address first?

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which response is accurate?

A. You arrived, so you consented to treatment.

B. The booking proves which tooth needs repair.

C. The record shows a consultation; the dentist can clarify the plan before you decide.

D. A thirty-minute slot guarantees a filling.

2. Which statement overpromises?

A. The dentist can address your question.

B. Your insurance will definitely pay because the visit is booked.

C. The estimate needs checking.

D. No procedure is confirmed yet.

3. A colleague needs the term for this meaning: The category of visit recorded in the schedule. Which term fits?

A. clinical decision

B. treatment option

C. appointment type

D. procedure code

D Choose the response that fits / continued

4. Rebuild the exchange from "A consultation is not a confirmed filling". Lena: "Does the shorter appointment mean the dentist has already decided there is nothing wrong? I do not want my concern dismissed before anyone looks." Amir: [missing response] Lena: "My main question is whether this tooth needs a filling at all. I also want to know what the alternatives would involve." Which response belongs in the gap?

A. That is a useful question about the treatment plan. I will make sure it reaches the dentist rather than promise that today's allocated time can accommodate any treatment that may be recommended.

B. No. The appointment duration does not establish a diagnosis or the seriousness of your concern. I will tell the dentist what you expected and ask them to clarify the purpose with you.

C. I can see why that is frustrating. I will flag the patient expectation to the dentist: you came for a filling, but our schedule shows a thirty-minute consultation. No procedure is confirmed yet.

D. I can pass on that chief concern. The dentist needs to assess and explain the options; I cannot determine the treatment from a booking entry or from where you point.

Answers and explanations, page 115

One useful distinction

A thirty-minute booking describes allocated time, not clinical suitability or consent for a filling. Explain the visit type and arrange the clinical discussion. Do not convert a hoped-for treatment into a confirmed procedure because the patient has arrived.

E Conversation

A consultation is not a confirmed filling

A consultation is not a confirmed filling

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

patient expectation / treatment estimate / decision time / coverage estimate / treatment plan / appointment type / Informed consent / chief concern / appointment duration / booking discrepancy

01 **Lena:** I am here for the filling. I took the afternoon off, so I hope we can finish it today rather than arrange another visit.

02 **Amir:** I can see why that is frustrating. I will flag the [1] _____ to the dentist: you came for a filling, but our schedule shows a thirty-minute consultation. No procedure is confirmed yet.

03 **Lena:** When I spoke to reception, I mentioned the tooth that was bothering me. I assumed booking an appointment meant booking the treatment as well.

04 **Amir:** The [2] _____ matters here. A consultation lets the dentist discuss the concern and appropriate next steps, but I should not tell you that it guarantees a filling today.

05 **Lena:** Does the shorter appointment mean the dentist has already decided there is nothing wrong? I do not want my concern dismissed before anyone looks.

06 **Amir:** No. The [3] _____ does not establish a diagnosis or the seriousness of your concern. I will tell the dentist what you expected and ask them to clarify the purpose with you.

E Conversation / continued

07 **Lena:** My main question is whether this tooth needs a filling at all. I also want to know what the alternatives would involve.

08 **Amir:** I can pass on that [4] _____. The dentist needs to assess and explain the options; I cannot determine the treatment from a booking entry or from where you point.

09 **Lena:** I would also like to understand the cost before agreeing. I received a number over the phone, but I do not know exactly what it covered.

10 **Amir:** We should clarify the [5] _____ and which service it described. A consultation estimate and an estimate for a particular restoration may not cover the same work.

Conversation continues on the next page.

A consultation is not a confirmed filling

CONVERSATION / CONTINUED

11 **Lena:** The receptionist also mentioned my insurer. Should I assume that the amount above that estimate will be paid by insurance?

12 **Amir:** No. A [6] _____ is not a payment guarantee. I can route the question to the appropriate person so the limits are clear before you rely on it.

13 **Lena:** If I stay to talk to the dentist, am I agreeing to go ahead with whatever they recommend during that conversation?

14 **Amir:** No. [7] _____ concerns the actual care being proposed and discussed. Attending this visit does not mean you have already accepted every possible procedure or cost.

15 **Lena:** Thank you. I want the dentist to know I expected treatment, but I do not want anyone to think I am demanding a procedure that might not be right.

16 **Amir:** I will describe the [8] _____ neutrally. Your expectation and your request to understand the options can both be included, without presenting either as a clinical decision.

17 **Lena:** Then I would like to ask the dentist to explain what can be decided today and whether another appointment might be needed.

18 **Amir:** That is a useful question about the [9] _____. I will make sure it reaches the dentist rather than promise that today's allocated time can accommodate any treatment that may be recommended.

A consultation is not a confirmed filling / continued

19 **Lena:** All right, I will speak to the dentist first. Please mention the time off work too. Another appointment would take some arranging.

20 **Amir:** I will. We will allow [10] _____ for that discussion and keep the current status clear: consultation booked, procedure unconfirmed, and your questions still to be answered.

CHECK THE GAPS

1. patient expectation / **2.** appointment type / **3.** appointment duration / **4.** chief concern / **5.** treatment estimate / **6.** coverage estimate / **7.** Informed consent / **8.** booking discrepancy / **9.** treatment plan / **10.** decision time

Why these answers fit, page 116

F Check accuracy and scope

A consultation is not a confirmed filling

Return to the case

Patient Lena arrives expecting a filling today. Assistant Amir sees a thirty-minute consultation on the schedule; no procedure is confirmed. The dentist is available to clarify the purpose before Lena decides whether to continue. Amir should acknowledge the time she has set aside, explain what the record actually shows, and preserve her questions about options, costs, and timing. He cannot promise a filling today, diagnose the tooth, or imply that attending a consultation means agreeing to whatever treatment might later be recommended.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What should be handed to the dentist?

A. An invented diagnosis

B. A statement that all treatment is already accepted

C. A changed record hiding the original booking

D. The mismatch between the expected filling and recorded consultation, plus the patient's questions

2. What does attendance establish by itself?

A. Confirmation of clinical suitability

B. Acceptance of every future cost

C. The patient came to the appointment, not agreement to every procedure

D. Consent to every possible treatment

Answers and explanations, page 118

Listen for the distinction

Repeat the exchange with the crown-fitting facts below. Keep the twenty-minute duration separate from readiness.

Correct the expectation without blaming the patient

A mismatch may result from several conversations or records. Explain what you can verify and invite clarification without declaring that the patient heard incorrectly. A neutral correction protects trust while leaving clinical decisions with the appropriate person.

F Say it / transfer to a new case

Clarify a review rather than a procedure booking

A patient expects a crown fitting. The schedule shows a twenty-minute review, and the dentist has not confirmed fitting readiness. The patient asks about timing and the estimate.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

twenty / review / crown fitting / unconfirmed

01 **Assistant:** The recorded appointment type is a [1] _____.

02 **Patient:** I expected a [2] _____.

03 **Assistant:** The allocated time is [3] _____ minutes.

04 **Assistant:** Fitting readiness remains [4] _____.

CHECK THE GAPS

1. review / **2.** crown fitting / **3.** twenty / **4.** unconfirmed

Why these answers fit, page 118

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "A consultation is not a confirmed filling". Lena: "If I stay to talk to the dentist, am I agreeing to go ahead with whatever they recommend during that conversation?" Amir: [missing response] Lena: "Thank you. I want the dentist to know I expected treatment, but I do not want anyone to think I am demanding a procedure that might not be right." Which response belongs in the gap?

A. No. Informed consent concerns the actual care being proposed and discussed. Attending this visit does not mean you have already accepted every possible procedure or cost.

B. I will describe the booking discrepancy neutrally. Your expectation and your request to understand the options can both be included, without presenting either as a clinical decision.

C. I will. We will allow decision time for that discussion and keep the current status clear: consultation booked, procedure unconfirmed, and your questions still to be answered.

D. We should clarify the treatment estimate and which service it described. A consultation estimate and an estimate for a particular restoration may not cover the same work.

Answers and explanations, page 119

A Read the situation

LESSON 02 / ROOM READINESS AND SAFE CLARIFICATION

Raise an unresolved instrument-readiness check clearly and seek a verified replacement without waiving the practice process.

> **The pack is present; readiness is not verified**
>
> Assistant Tessa sees instrument pack P18 in the treatment room. The required readiness documentation cannot be confirmed, and the dentist is about to begin. Tessa has not released the pack for use and is not authorized to waive the practice checks. Lead assistant Bruno can investigate and may have a verified replacement. No contamination or sterilization failure has been established in the supplied facts. The immediate communication task is to make the unresolved check visible, keep the pack from being used until properly resolved, and follow the actual practice procedure.

People: Tessa, Dental assistant; Bruno, Lead assistant.

Read the record

Choose one answer per question using the facts above.

1. What is known about P18?

A. Every required check has passed.
B. It was already used on the patient.
C. It is present, but required readiness documentation cannot be confirmed.
D. It has been proved contaminated.

2. What may Bruno provide?

A. A guaranteed replacement before checking
B. A replacement whose readiness has been verified
C. An automatic waiver for every pack
D. Permission inferred from the busy schedule

3. What must not happen?

A. Reporting the gap
B. Asking for clarification
C. Identifying P18 in the record
D. Treating pack presence as permission to skip the required checks

Answers and explanations, page 120

B Vocabulary in use

CORE TERMS

instrument pack
A packaged set of dental instruments with a defined processing and use status.

identify the instrument pack

reprocessing
The required sequence for preparing a reusable device for further use.

follow reprocessing instructions

decontamination
Removal or reduction of contamination through the applicable process.

follow decontamination procedures

cleaning
Removal of soil and material, distinct from sterilization.

verify the cleaning step

disinfection
A process reducing or inactivating specified microorganisms under defined conditions.

distinguish disinfection from sterilization

sterilization
A validated process intended to eliminate viable microorganisms, including spores.

verify sterilization records

sterilizer
Equipment used for an appropriate sterilization process.

identify the sterilizer

cycle record
Documentation of a particular processing cycle.

trace the cycle record

load number
The identifier linking items to a particular processing load.

verify the load number

mechanical monitoring
Checking process parameters such as time, temperature, and pressure as applicable.

review mechanical monitoring

chemical indicator
A material showing exposure to specified process conditions.

inspect the chemical indicator

biological indicator
A test system using resistant microorganisms to assess the sterilization process.

review biological indicator results

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

instrument pack INSTRUMENT PACK

identify the instrument pack

Keep the meaning precise: A packaged set of dental instruments with a defined processing and use status.

sterilization STERILIZATION

verify sterilization records

Keep the meaning precise: A validated process intended to eliminate viable microorganisms, including spores.

B Precision and professional shorthand

EXTEND YOUR RANGE

pack integrity

The condition of the packaging and its protective barrier.

check pack integrity

compromised packaging

Packaging whose protective condition has been damaged or otherwise impaired.

report compromised packaging

IFU

Instructions for use; the relevant manufacturer's directions.

consult the IFU

single-use device

A device intended for one use under its labeling.

identify a single-use device

reusable device

A device intended for further use after the specified reprocessing.

identify a reusable device

readiness documentation

Records needed to establish the relevant item is ready under the procedure.

confirm readiness documentation

traceability

Ability to connect an item with relevant identity and processing history.

maintain traceability

release status

Whether an item is authorized for use under the required checks.

confirm release status

hold

A status preventing use pending the appropriate resolution.

maintain the hold

verified replacement

An alternative item whose required readiness has been confirmed.

obtain a verified replacement

processing deviation

A departure from the specified processing requirement.

report a processing deviation

infection prevention

Measures intended to reduce the risk of infection transmission.

follow infection prevention procedures

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

biological indicator BIOLOGICAL INDICATOR

review biological indicator results

Keep the meaning precise: A test system using resistant microorganisms to assess the sterilization process.

reusable device REUSABLE DEVICE

identify a reusable device

Keep the meaning precise: A device intended for further use after the specified reprocessing.

C Phrases that move the work forward

Interrupt clearly

Please pause; I cannot confirm the required readiness check for P18.

Identify the item

The concern is about pack P18 in this room.

State the gap

The required documentation is not available for verification.

Avoid an unsupported finding

I am not reporting a confirmed contamination event.

Prevent an assumption

The pack being present does not establish its release status.

Name the limit

I am not authorized to waive this check.

Request the lead

Can you review the status through the practice procedure?

Ask about an alternative

Is a verified replacement available?

Keep a hold visible

This pack should not be used while the required check remains unresolved.

Preserve traceability

Keep the pack identifier linked to the review record.

Clarify the evidence

Which record confirms the relevant processing and release checks?

Reject schedule pressure

The appointment timing does not complete the missing check.

Confirm the replacement

Please confirm the replacement identifier and verified status.

Separate two outcomes

Replacing the pack does not by itself resolve the original documentation issue.

Record the action

I will record the concern and the action actually taken.

Close the loop

We need an explicit readiness confirmation before describing the setup as ready.

Precision in real exchanges

Purpose: Avoid an unsupported finding
Useful wording: I am not reporting a confirmed contamination event.

Purpose: Prevent an assumption
Useful wording: The pack being present does not establish its release status.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which message is best?

A. The pack looks clean, so it must be ready.

B. Every instrument is contaminated, without evidence.

C. We can waive the check because the dentist is waiting.

D. Please pause; P18's required readiness record is unconfirmed, and I need the approved review or a verified replacement.

2. What does a color change alone establish here?

A. That pack integrity no longer needs checking

B. Not completion of every required readiness check

C. That the pack's release record is complete

D. That the biological monitoring result has passed

3. A colleague needs the term for this meaning: Removal of soil and material, distinct from sterilization. Which term fits?

A. cleaning

B. load number

C. IFU

D. hold

D Choose the response that fits / continued

4. Rebuild the exchange from "The pack is present; readiness is not verified". Tessa: "The package is here, but I do not want to say it is contaminated. The issue I can establish is that the required documentation cannot be confirmed." Bruno: [missing response] Tessa: "Would a replacement solve the immediate setup problem? I heard there might be another pack, but I have not checked its identity or status." Which response belongs in the gap?

A. And follow the relevant IFU and practice procedures for the item. Our handoff should end with explicit confirmed facts, not with the assumption that someone else must have completed the check.

B. Then keep the release status unresolved under our procedure. I will help review the record rather than treat the presence of the pack as proof that it is ready.

C. A chemical indicator has a defined purpose, but it is not a substitute for all applicable checks. We need the practice process and manufacturer instructions followed, not an isolated shortcut.

D. That is the right distinction. We need the readiness documentation, not an invented finding of failure or a reassuring assumption. The uncertainty still means we must not bypass the check.

Answers and explanations, page 120

> **One useful distinction**
>
> Present, packaged, clean, disinfected, and sterile are not interchangeable statuses. Recognition of an indicator or a sealed pack does not replace the required combined checks, records, and manufacturer directions. This unit practices clarification, not a processing protocol.

E Conversation

The pack is present; readiness is not verified

The pack is present; readiness is not verified

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

chemical indicator / instrument pack / readiness documentation / load number / Traceability / verified replacement / hold / cycle record / release status / IFU

01 **Tessa:** Bruno, please pause the setup. Pack P18 is present, but I cannot confirm its required readiness documentation, and the dentist is about to begin.

02 **Bruno:** P18, understood. Has that [1] _____ been opened or used? Tell me where it is now so nobody picks it up while I check the record.

03 **Tessa:** It has not been used. I have not released it, and I do not have authority to waive the missing check just because the room is busy.

04 **Bruno:** Then keep the [2] _____ unresolved under our procedure. I will help review the record rather than treat the presence of the pack as proof that it is ready.

05 **Tessa:** The package is here, but I do not want to say it is contaminated. The issue I can establish is that the required documentation cannot be confirmed.

06 **Bruno:** That is the right distinction. We need the [3] _____, not an invented finding of failure or a reassuring assumption. The uncertainty still means we must not bypass the check.

07 **Tessa:** Would a replacement solve the immediate setup problem? I heard there might be another pack, but I have not checked its identity or status.

08 **Bruno:** A [4] _____ may be available. I will check the actual alternative and its required status, not simply swap one unverified package for another because it is nearby.

09 **Tessa:** If we locate the original record, we also need to know that it belongs to P18 rather than a similar pack from another processing load.

10 **Bruno:** Correct. [5] _____ connects the item to the relevant history. A complete-looking record for a different load would not answer the question about this pack.

Conversation continues on the next page.

The pack is present; readiness is not verified

CONVERSATION / CONTINUED

11 **Tessa:** I can give you the identifiers shown without altering anything. I do not want to copy a number from another pack into the missing record.

12 **Bruno:** Preserve the actual [6] _____ and follow the documentation process. A guessed identifier would hide the problem rather than establish that the required processing and checks occurred.

13 **Tessa:** Someone asked whether the visible indicator was enough. I was unsure how to explain why one visible feature did not settle every readiness question.

14 **Bruno:** A [7] _____ has a defined purpose, but it is not a substitute for all applicable checks. We need the practice process and manufacturer instructions followed, not an isolated shortcut.

15 **Tessa:** I will tell the dentist that I have raised the specific gap and that you are checking the record and a possible verified alternative.

16 **Bruno:** Keep the [8] _____ visible. I am checking the replacement now; please tell the dentist that setup is still waiting for verification, not ready yet.

17 **Tessa:** If another pack is confirmed, should I describe the entire issue as closed, or distinguish the replacement from the original record question?

18 **Bruno:** Distinguish them. The [9] _____ issue may require further review even after the immediate setup has an acceptable alternative. Do not erase the first concern from the history.

19 **Tessa:** Understood. I will record the actual pack status, whom I contacted, and any verified replacement details, without claiming a failure that has not been found.

20 **Bruno:** And follow the relevant [10] _____ and practice procedures for the item. Our handoff should end with explicit confirmed facts, not with the assumption that someone else must have completed the check.

CHECK THE GAPS

1. instrument pack / **2.** release status / **3.** readiness documentation / **4.** verified replacement / **5.** Traceability / **6.** load number / **7.** chemical indicator / **8.** hold / **9.** cycle record / **10.** IFU

Why these answers fit, page 122

F Check accuracy and scope

The pack is present; readiness is not verified

Return to the case

Assistant Tessa sees instrument pack P18 in the treatment room. The required readiness documentation cannot be confirmed, and the dentist is about to begin. Tessa has not released the pack for use and is not authorized to waive the practice checks. Lead assistant Bruno can investigate and may have a verified replacement. No contamination or sterilization failure has been established in the supplied facts. The immediate communication task is to make the unresolved check visible, keep the pack from being used until properly resolved, and follow the actual practice procedure.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What should happen to the original issue if a replacement is found?

A. It automatically disappears from all records.
B. It proves P18 was contaminated.
C. It permits changing the old record to show prior verification.
D. It remains subject to the required review and record process.

2. Which statement is precise?

A. Missing records prove every cycle failed.
B. No documented failure means automatic permission to use the pack.
C. Readiness is unverified; no confirmed failure is established in the supplied facts.
D. Readiness and contamination are identical words.

Answers and explanations, page 123

Listen for the distinction

Repeat the four-line transfer using its different pack and check; do not call an unresolved check a confirmed contamination event.

A clear interruption can protect the workflow

A junior worker may hesitate when the team is ready to begin. Name the specific unresolved check and the needed action. A factual concern is not a personal accusation, and politeness should not make a request to pause sound optional.

F Say it / transfer to a new case

Identify a different unresolved readiness check

Pack Q7 has not been used. Its required readiness check is unconfirmed. Lead assistant Eva can review it; a replacement has been requested but not yet verified.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

Q7 / Eva / verified / not used

01 **Assistant:** The affected pack is [1] _____.

02 **Lead:** Its use status is [2] _____.

03 **Assistant:** The review contact is [3] _____.

04 **Lead:** The replacement is requested but not yet [4] _____.

CHECK THE GAPS

1. Q7 / **2.** not used / **3.** Eva / **4.** verified

Why these answers fit, page 123

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "The pack is present; readiness is not verified". Tessa: "Someone asked whether the visible indicator was enough. I was unsure how to explain why one visible feature did not settle every readiness question." Bruno: [missing response] Tessa: "I will tell the dentist that I have raised the specific gap and that you are checking the record and a possible verified alternative." Which response belongs in the gap?

A. A chemical indicator has a defined purpose, but it is not a substitute for all applicable checks. We need the practice process and manufacturer instructions followed, not an isolated shortcut.

B. That is the right distinction. We need the readiness documentation, not an invented finding of failure or a reassuring assumption. The uncertainty still means we must not bypass the check.

C. Then keep the release status unresolved under our procedure. I will help review the record rather than treat the presence of the pack as proof that it is ready.

D. And follow the relevant IFU and practice procedures for the item. Our handoff should end with explicit confirmed facts, not with the assumption that someone else must have completed the check.

Answers and explanations, page 124

Review 1 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Clarifying the appointment purpose" and "Room readiness and safe clarification". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Material placed to restore a prepared tooth area as clinically indicated. Which term fits?

A. restorative dentistry
B. treatment plan
C. booking discrepancy
D. filling

2. A colleague needs the term for this meaning: Documentation of a particular processing cycle. Which term fits?

A. pack integrity
B. instrument pack
C. cycle record
D. traceability

3. Rebuild the exchange from "A consultation is not a confirmed filling". Lena: "I would also like to understand the cost before agreeing. I received a number over the phone, but I do not know exactly what it covered." Amir: [missing response] Lena: "The receptionist also mentioned my insurer. Should I assume that the amount above that estimate will be paid by insurance?" Which response belongs in the gap?

A. I will describe the booking discrepancy neutrally. Your expectation and your request to understand the options can both be included, without presenting either as a clinical decision.
B. We should clarify the treatment estimate and which service it described. A consultation estimate and an estimate for a particular restoration may not cover the same work.
C. No. A coverage estimate is not a payment guarantee. I can route the question to the appropriate person so the limits are clear before you rely on it.
D. I will. We will allow decision time for that discussion and keep the current status clear: consultation booked, procedure unconfirmed, and your questions still to be answered.

Review 1 / Meaning and context / continued

4. Rebuild the exchange from "The pack is present; readiness is not verified". Tessa: "If another pack is confirmed, should I describe the entire issue as closed, or distinguish the replacement from the original record question?" Bruno: [missing response] Tessa: "Understood. I will record the actual pack status, whom I contacted, and any verified replacement details, without claiming a failure that has not been found." Which response belongs in the gap?

A. Distinguish them. The cycle record issue may require further review even after the immediate setup has an acceptable alternative. Do not erase the first concern from the history.

B. A verified replacement may be available. I will check the actual alternative and its required status, not simply swap one unverified package for another because it is nearby.

C. Preserve the actual load number and follow the documentation process. A guessed identifier would hide the problem rather than establish that the required processing and checks occurred.

D. Then keep the release status unresolved under our procedure. I will help review the record rather than treat the presence of the pack as proof that it is ready.

Answers and explanations, page 155

A Read the situation

LESSON 03 / CHAIRSIDE REQUESTS AND READ-BACKS

Identify the word you missed, request the distinguishing detail, and confirm the full chairside instruction before acting.

> **Which tray did you mean?**
>
> During a setup briefing, assistant Noor hears Dr. Shaw say "tray" but misses the rest because another conversation interrupts. The setup list contains both an instrument tray and an impression tray. Noor has not selected either. She needs to ask which item is intended and read back the complete request before continuing under practice procedures. The dentist then clarifies that the instrument tray is wanted for the current setup. Knowing the general treatment context does not justify guessing the missing instruction, and this language exercise does not authorize a clinical procedure.

People: Noor, Dental assistant; Dr. Shaw, Dentist.

Read the record

Choose one answer per question using the facts above.

1. Which word did Noor hear?

A. A confirmed size
B. Tray
C. Instrument
D. Impression

2. What are the two listed possibilities?

A. Instrument tray and impression tray
B. Two confirmed identical instruments
C. A radiograph and a consent form
D. Two appointment times

3. What does the dentist clarify?

A. Noor should guess each future request.
B. The impression tray was already selected.
C. Every clinical check is waived.
D. The instrument tray is wanted for the current setup.

Answers and explanations, page 125

B Vocabulary in use

CORE TERMS

chairside
Relating to assistance beside the dental chair during care.

provide chairside support

instrument tray
A tray used to organize the relevant dental instruments.

confirm the instrument tray

impression
A negative reproduction used in making a model of oral structures.

distinguish an impression from a scan

mirror
A dental instrument used for viewing or related clinical purposes.

identify the requested mirror

periodontal probe
An instrument for periodontal measurements under the relevant clinical procedure.

distinguish the periodontal probe

HVE
High-volume evacuation; a dental suction system or device.

clarify the HVE request

setup briefing
A discussion confirming the planned equipment and arrangements.

conduct a setup briefing

impression tray
A device used to hold impression material under the appropriate procedure.

identify the impression tray

intraoral scan
A digital capture of structures inside the mouth.

identify the intraoral scan

explorer
A dental examination instrument used by appropriately trained personnel.

identify the explorer

cotton pliers
An instrument for handling suitable small materials.

identify the cotton pliers

saliva ejector
A suction device intended for removing saliva under its specified use.

distinguish the saliva ejector

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

chairside CHAIRSIDE

provide chairside support

Keep the meaning precise: Relating to assistance beside the dental chair during care.

intraoral scan INTRAORAL SCAN

identify the intraoral scan

Keep the meaning precise: A digital capture of structures inside the mouth.

B Precision and professional shorthand

EXTEND YOUR RANGE

air-water syringe

A dental device delivering air, water, or a combination under its design.

identify the air-water syringe

bur

A rotary cutting or finishing attachment with a particular design and purpose.

verify the bur identifier

wedge

A small device used for specified separation or adaptation tasks in dentistry.

clarify the requested wedge

retractor

An instrument or device used to hold tissue away as appropriate.

identify the retractor

read-back

Repeating an instruction to check it was heard correctly.

give a complete read-back

ambiguous instruction

Wording that supports more than one interpretation.

clarify an ambiguous instruction

handpiece

A powered dental instrument used with specified attachments.

clarify the handpiece type

matrix band

A device used to provide a temporary wall during certain restorations.

identify the matrix band

curing light

A device used to initiate setting of compatible dental materials.

identify the curing light

distinguishing detail

The feature that separates one possible request from another.

request the distinguishing detail

closed-loop request

A request followed by acknowledgment and confirmation of the intended information.

complete a closed-loop request

interruption

Something that breaks attention or prevents hearing the full message.

acknowledge an interruption

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

saliva ejector SALIVA EJECTOR

distinguish the saliva ejector

Keep the meaning precise: A suction device intended for removing saliva under its specified use.

wedge WEDGE

clarify the requested wedge

Keep the meaning precise: A small device used for specified separation or adaptation tasks in dentistry.

C Phrases that move the work forward

Signal the gap

I heard tray, but I missed the word before it.

Name the interruption

Another conversation covered the rest of the request.

Offer the alternatives

Did you mean the instrument tray or the impression tray?

Avoid guessing

I have not selected either yet.

Ask for the whole request

Could you repeat the full item name?

Read back

The instrument tray for the current setup, correct?

Check a modifier

Which size or identifier did you specify?

Clarify sequence

Do you need that now or at the next stage?

Confirm a correction

Thank you; I will use the corrected request.

Avoid vague agreement

I need to confirm the item before saying it is ready.

Keep checks separate

Hearing the request correctly does not replace the readiness checks.

Name unfamiliar shorthand

I do not know that abbreviation in this practice.

Request ordinary wording

Could you say the full name once?

State what is confirmed

We have confirmed the instrument tray, not the impression tray.

Protect the handoff

I will pass on the complete request rather than the shortened version.

Close the loop

I have repeated the item, and you have confirmed it.

Precision in real exchanges

Purpose: Avoid guessing
Useful wording: I have not selected either yet.

Purpose: Ask for the whole request
Useful wording: Could you repeat the full item name?

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which question best resolves the actual ambiguity?

A. Can I ignore the rest because I heard one word?

B. Did you mean the instrument tray or the impression tray?

C. You said tray, so either must be acceptable.

D. Shall I choose the one nearest me?

2. Which read-back is complete for the clarified request?

A. The instrument tray for the current setup, correct?

B. The tray from yesterday, correct?

C. The impression tray for the current setup, correct?

D. The instrument tray for the next appointment, correct?

3. A colleague needs the term for this meaning: A device used to hold impression material under the appropriate procedure. Which term fits?

A. impression tray

B. bur

C. periodontal probe

D. read-back

D Choose the response that fits / continued

4. Rebuild the exchange from "Which tray did you mean?". Noor: "I heard suction from the doorway earlier. Do you mean the HVE rather than the saliva ejector? Both are on the equipment list." Dr. Shaw: [missing response] Noor: "HVE confirmed. Marta has asked which tray to bring. I will give her the full item name and tell her this is for the current setup." Which response belongs in the gap?

A. Correct: no impression tray in this request. That is about our current setup, not a change to another patient's appointment or planned equipment.

B. That completes the closed-loop request. Let me know if the check is unresolved; an accurate equipment name does not settle its processing status.

C. Correct; that read-back matches. Also, have the high-volume evacuator ready under our normal checks. We will confirm the remaining equipment during the briefing.

D. Yes, HVE. That is the distinguishing detail I left out earlier. Please keep those names separate when you pass the request to Marta.

Answers and explanations, page 125

> **One useful distinction**
>
> Instrument tray and impression tray share a word but identify different items. Repeat the distinguishing term, not only tray. The same principle applies to size, identifier, side, and timing when those details are part of the actual request.

E Conversation

Which tray did you mean?

Which tray did you mean?

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

interruption / setup briefing / handoff / instrument tray / read-back / closed-loop request / impression tray / ambiguous instruction / chairside / distinguishing detail

01 **Noor:** Dr. Shaw, I heard tray, but the conversation by the door covered the first word. Instrument tray or impression tray? I have not selected either.

02 **Dr. Shaw:** Sorry about the [1] _____. The instrument tray for this setup, please. The impression tray is not part of this request.

03 **Noor:** Instrument tray for the current setup, correct? I will check its readiness status before bringing it over. The other tray will stay where it is.

04 **Dr. Shaw:** Correct; that [2] _____ matches. Also, have the high-volume evacuator ready under our normal checks. We will confirm the remaining equipment during the briefing.

05 **Noor:** I heard suction from the doorway earlier. Do you mean the HVE rather than the saliva ejector? Both are on the equipment list.

06 **Dr. Shaw:** Yes, HVE. That is the [3] _____ I left out earlier. Please keep those names separate when you pass the request to Marta.

07 **Noor:** HVE confirmed. Marta has asked which tray to bring. I will give her the full item name and tell her this is for the current setup.

08 **Dr. Shaw:** Ask her for the [4] _____, with the required readiness checks completed. If she cannot confirm its status, have her tell us before it reaches the setup.

09 **Noor:** One more point: someone put small suction on the handwritten list. Is that your request, or a note carried over from the previous appointment?

10 **Dr. Shaw:** I cannot tell from that note. Treat it as an [5] _____, not a substitute for the HVE request we have just confirmed. I will check its source.

Conversation continues on the next page.

Which tray did you mean?

CONVERSATION / CONTINUED

11 **Noor:** I will mark that question as unresolved. I will not remove an equipment requirement merely because the handwriting is unclear. We still need to establish what it refers to.

12 **Dr. Shaw:** During the [6] _____, we will reconcile that note with the actual list. Please bring the original wording so I can see what needs clarification.

13 **Noor:** For any size-specific request, please include the size in the full name. I missed a modifier in yesterday's briefing and had to ask again.

14 **Dr. Shaw:** Of course. In a [7] _____ exchange, say which part you missed. I would rather repeat one size than discover that we understood different items.

15 **Noor:** Marta is here now. She has repeated instrument tray and HVE. She also wants to confirm that you did not request impression equipment at this stage.

16 **Dr. Shaw:** Correct: no [8] _____ in this request. That is about our current setup, not a change to another patient's appointment or planned equipment.

17 **Noor:** I have repeated that to Marta, and she has confirmed it back. She is checking the tray's readiness and will report before bringing it across.

18 **Dr. Shaw:** That completes the [9] _____. Let me know if the check is unresolved; an accurate equipment name does not settle its processing status.

19 **Noor:** My note reads: instrument tray and HVE for this setup, checks required; small suction note awaiting clarification. Marta has those same details.

20 **Dr. Shaw:** That is the [10] _____ I need. Bring the handwritten list to the briefing, and we will settle the remaining point before relying on it.

CHECK THE GAPS

1. interruption / **2.** read-back / **3.** distinguishing detail / **4.** instrument tray / **5.** ambiguous instruction / **6.** setup briefing / **7.** chairside / **8.** impression tray / **9.** closed-loop request / **10.** handoff

Why these answers fit, page 126

F Check accuracy and scope

Which tray did you mean?

Return to the case

During a setup briefing, assistant Noor hears Dr. Shaw say "tray" but misses the rest because another conversation interrupts. The setup list contains both an instrument tray and an impression tray. Noor has not selected either. She needs to ask which item is intended and read back the complete request before continuing under practice procedures. The dentist then clarifies that the instrument tray is wanted for the current setup. Knowing the general treatment context does not justify guessing the missing instruction, and this language exercise does not authorize a clinical procedure.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What should Noor do with unfamiliar local shorthand?

A. Pretend every abbreviation is universal.
B. Choose an item based only on its color.
C. Change the request without telling anyone.
D. Ask for the full name and confirm the intended meaning.

2. What does confirmation of the request establish?

A. A waiver of the practice procedure
B. Consent for an unrelated treatment
C. Shared understanding of the requested item, not completion of all readiness checks
D. Universal competence to use all devices

Answers and explanations, page 127

Listen for the distinction

Repeat the suction-device transfer, clearly distinguishing HVE from saliva ejector.

A brief clarification is professional

Fast speech and local shorthand can make an unfamiliar term seem obvious to everyone else. State exactly what you heard and what you missed. A precise question is more useful than a vague yes, an apology repeated several times, or a silent guess.

F Say it / transfer to a new case

Clarify a suction-device request

During a briefing, an assistant hears suction but misses the device name. The list includes an HVE device and a saliva ejector. The dentist clarifies HVE for the current setup; normal checks remain required.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

required / read-back / device name / HVE

01 **Assistant:** I heard suction but missed the [1] _____.

02 **Dentist:** The requested device is [2] _____.

03 **Assistant:** I will confirm it with a complete [3] _____.

04 **Dentist:** The ordinary readiness checks remain [4] _____.

CHECK THE GAPS

1. device name / **2.** HVE / **3.** read-back / **4.** required

Why these answers fit, page 128

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Which tray did you mean?". Noor: "For any size-specific request, please include the size in the full name. I missed a modifier in yesterday's briefing and had to ask again." Dr. Shaw: [missing response] Noor: "Marta is here now. She has repeated instrument tray and HVE. She also wants to confirm that you did not request impression equipment at this stage." Which response belongs in the gap?

A. Of course. In a chairside exchange, say which part you missed. I would rather repeat one size than discover that we understood different items.

B. Correct; that read-back matches. Also, have the high-volume evacuator ready under our normal checks. We will confirm the remaining equipment during the briefing.

C. That is the handoff I need. Bring the handwritten list to the briefing, and we will settle the remaining point before relying on it.

D. Yes, HVE. That is the distinguishing detail I left out earlier. Please keep those names separate when you pass the request to Marta.

Answers and explanations, page 129

A Read the situation

LESSON 04 / IMAGING QUESTIONS AND AUTHORIZATION

Acknowledge a question about dental imaging and arrange clinical explanation without interpreting findings or promising treatment.

> **Explain the route to an answer, not the image**
>
> Patient Marco looks at a dental image and asks assistant Suki whether it proves that a tooth needs treatment. The dentist has not yet discussed the image with him. Suki is assisting with the visit and is not authorized to interpret the image or recommend treatment. She can identify the question, explain her role, and ask the dentist to discuss the findings and their significance. No finding, diagnosis, or treatment decision is supplied in this case. Technical vocabulary supports communication; it does not qualify a learner to read an image.

People: Marco, Patient; Suki, Dental assistant.

Read the record

Choose one answer per question using the facts above.

1. What does Marco ask?

A. Whether the image proves that a tooth needs treatment
B. Whether the laboratory confirmed delivery
C. Which pack was processed
D. Whether his appointment was cancelled

2. What is Suki not authorized to do here?

A. Preserve the patient's concern
B. Interpret the image or recommend treatment
C. Acknowledge the question
D. Ask the dentist to explain

3. What clinical finding is established in the brief?

A. A definite need for extraction
B. None
C. A confirmed cavity
D. A confirmed fracture

Answers and explanations, page 130

B Vocabulary in use

CORE TERMS

radiograph
An image produced using X-rays.

identify the radiograph

radiography
The process of producing radiographic images under the applicable rules.

distinguish radiography from interpretation

intraoral image
In dental radiography, an image obtained with the receptor inside the mouth.

identify an intraoral image

extraoral image
In dental radiography, an image obtained with the receptor outside the mouth.

identify an extraoral image

bitewing
A dental radiographic view commonly showing crowns of upper and lower teeth and nearby structures.

identify the bitewing view

periapical image
A dental image showing a tooth and the area around its root tip.

identify a periapical image

panoramic image
A broad dental image of the jaws and surrounding structures.

identify a panoramic image

CBCT
Cone-beam computed tomography; three-dimensional X-ray imaging used for appropriate indications.

identify the CBCT study

image receptor
The film or digital sensor receiving the image information.

identify the image receptor

field of view
The area or volume included in an imaging examination.

clarify the field of view

exposure
The use of radiation to obtain an image under an authorized process.

document the exposure

image quality
Whether an image meets the relevant technical requirements for its intended use.

assess image quality within role

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

radiograph RADIOGRAPH

identify the radiograph

Keep the meaning precise: An image produced using X-rays.

periapical image PERIAPICAL IMAGE

identify a periapical image

Keep the meaning precise: A dental image showing a tooth and the area around its root tip.

B Precision and professional shorthand

EXTEND YOUR RANGE

artifact
An image feature introduced by a technical or other non-anatomical factor.

report a possible artifact

radiopaque
Appearing relatively lighter on a radiograph because fewer X-rays reach the receptor.

recognize the term radiopaque

image interpretation
Professional assessment of what an image shows and means.

refer image interpretation

diagnostic significance
The relevance of information to identifying a condition.

ask about diagnostic significance

comparison image
An earlier or other image used for a defined comparison.

identify the comparison image

treatment indication
The clinical reason for considering a particular treatment.

explain the treatment indication

radiolucent
Appearing relatively darker on a radiograph because more X-rays reach the receptor.

recognize the term radiolucent

anatomical structure
A part of the body represented or examined.

identify the named anatomical structure

clinical finding
Information established through the appropriate clinical assessment.

explain a clinical finding

correlation
Considering information together with other relevant findings.

correlate clinical information

image annotation
A label or mark added to an image with its meaning and source identified.

clarify an image annotation

authorized interpreter
A professional permitted and qualified to interpret the relevant image.

refer to the authorized interpreter

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

image quality IMAGE QUALITY

assess image quality within role

Keep the meaning precise: Whether an image meets the relevant technical requirements for its intended use.

image interpretation IMAGE INTERPRETATION

refer image interpretation

Keep the meaning precise: Professional assessment of what an image shows and means.

C Phrases that move the work forward

Acknowledge the question

You want to know whether the image means this tooth needs treatment.

Offer the next step

I will ask the dentist to explain the findings with you.

Avoid false reassurance

I should not tell you everything is normal without the clinical explanation.

Preserve the exact concern

I will include your question about whether treatment is necessary.

Clarify an annotation

Please ask the clinician what that marked area refers to.

Invite evidence questions

You can ask how the image relates to the rest of the examination.

Avoid overstating proof

I cannot say the image alone proves a particular treatment is needed.

Check the handoff

I will make sure the question reaches the dentist before you decide.

State the role

I am not authorized to interpret the image.

Avoid a visual guess

I cannot identify a diagnosis from that feature for you.

Separate process and meaning

Helping with an image is different from interpreting it.

Ask for ordinary language

Could the dentist explain that term in everyday language?

Keep decisions open

No treatment decision has been explained in this conversation yet.

Distinguish alternatives

You can ask which options are being considered and why.

Route a technical question

The clinician can explain the significance of that imaging term.

Close without abandonment

I cannot give the interpretation, but I can help you get a clear answer.

Precision in real exchanges

Purpose: Avoid a visual guess
Useful wording: I cannot identify a diagnosis from that feature for you.

Purpose: Avoid false reassurance
Useful wording: I should not tell you everything is normal without the clinical explanation.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which response is appropriate?

A. Everything is normal because you are not in pain.

B. Any image means treatment is compulsory.

C. I cannot interpret it, but I will ask the dentist to explain whether it supports treatment and why.

D. The dark area definitely proves decay.

2. What is wrong with labeling an unexplained feature a cavity?

A. The patient has already received an explanation.

B. The schedule supplies diagnostic proof.

C. No interpreted finding is supplied, and Suki lacks authorization to diagnose it.

D. Every dark feature always means the same thing.

3. A colleague needs the term for this meaning: In dental radiography, an image obtained with the receptor outside the mouth. Which term fits?

A. extraoral image

B. radiopaque

C. image receptor

D. comparison image

D Choose the response that fits / continued

4. Rebuild the exchange from "Explain the route to an answer, not the image". Marco: "Is darker always worse? I have heard words used for dark and light areas, but I do not know whether they describe a disease." Suki: [missing response] Marco: "Then I should ask what the feature actually represents. I would also like to know whether anything else in the examination supports the same conclusion." Which response belongs in the gap?

A. Terms such as radiolucent describe image appearance, not a diagnosis by themselves. The dentist can explain the relevant terminology in context rather than have us attach a condition to an isolated word.

B. The clinician can clarify that image annotation and what it refers to. I should not guess the purpose of a mark or present it as proof of a particular condition.

C. That is a question about correlation with other clinical information. I will pass it on along with your question about whether a treatment recommendation is supported and what alternatives exist.

D. You are asking about diagnostic significance. I am not authorized to interpret the image, but I will ask the dentist to explain the finding, if any, and how it relates to treatment.

Answers and explanations, page 130

> **One useful distinction**
>
> Acquiring, labeling, displaying, and interpreting an image are different activities. The actual role and local rules determine who may do each. A visible feature or a technical term does not establish a diagnosis or make an assistant authorized to provide one.

E Conversation

Explain the route to an answer, not the image

Explain the route to an answer, not the image

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

correlation / treatment indication / authorized interpreter / radiolucent / image interpretation / artifact / radiograph / comparison image / image annotation / diagnostic significance

01 **Marco:** Suki, I can see a darker area on this picture. Does that prove the tooth needs treatment, or am I misunderstanding what I am looking at?

02 **Suki:** You are asking about [1] _____. I am not authorized to interpret the image, but I will ask the dentist to explain the finding, if any, and how it relates to treatment.

03 **Marco:** I thought you could explain it because you displayed it. Who should I ask about that darker patch?

04 **Suki:** Displaying a [2] _____ and interpreting it are different tasks. I can help communicate the question without giving you a diagnosis from something I see on the display.

05 **Marco:** Is darker always worse? I have heard words used for dark and light areas, but I do not know whether they describe a disease.

06 **Suki:** Terms such as [3] _____ describe image appearance, not a diagnosis by themselves. The dentist can explain the relevant terminology in context rather than have us attach a condition to an isolated word.

E Conversation / continued

07 **Marco:** Then I should ask what the feature actually represents. I would also like to know whether anything else in the examination supports the same conclusion.

08 **Suki:** That is a question about [4] _____ with other clinical information. I will pass it on along with your question about whether a treatment recommendation is supported and what alternatives exist.

09 **Marco:** There is also a mark on the screen. I cannot tell whether it is part of the original image or something someone added to point out an area.

10 **Suki:** The clinician can clarify that [5] _____ and what it refers to. I should not guess the purpose of a mark or present it as proof of a particular condition.

Conversation continues on the next page.

Explain the route to an answer, not the image

CONVERSATION / CONTINUED

11 **Marco:** Could it be a technical issue instead? I am not asking you to decide that, but I would like the dentist to address the possibility.

12 **Suki:** I will include your question about a possible [6] _____. We should keep that as a question, not replace one unsupported conclusion with another.

13 **Marco:** I appreciate that. I do not want reassurance if it is not justified, but I also do not want a frightening answer based on a guess.

14 **Suki:** The [7] _____ can explain what is established and what remains uncertain. I can make sure your concern is heard without telling you that everything is normal or definitely abnormal.

15 **Marco:** If a treatment is recommended, I want to understand why that treatment follows from the evidence rather than think the picture makes the decision automatically.

16 **Suki:** I will ask the dentist to explain the [8] _____ in ordinary language. An image on a screen is not a substitute for the discussion you need before deciding.

17 **Marco:** Would an earlier image matter? Has the dentist compared this with my previous image?

18 **Suki:** You can ask whether a [9] _____ was relevant and reviewed. I will not say that comparison has happened unless it is confirmed in the actual explanation.

19 **Marco:** Please ask what the patch means and whether it matches the examination. That is what I need to understand first.

20 **Suki:** I will preserve them for the [10] _____ discussion. We have identified the questions, not a diagnosis or an agreed procedure, and I will help you obtain the clinical explanation.

CHECK THE GAPS

1. diagnostic significance / **2.** radiograph / **3.** radiolucent / **4.** correlation / **5.** image annotation / **6.** artifact / **7.** authorized interpreter / **8.** treatment indication / **9.** comparison image / **10.** image interpretation

Why these answers fit, page 132

F Check accuracy and scope

Explain the route to an answer, not the image

Return to the case

Patient Marco looks at a dental image and asks assistant Suki whether it proves that a tooth needs treatment. The dentist has not yet discussed the image with him. Suki is assisting with the visit and is not authorized to interpret the image or recommend treatment. She can identify the question, explain her role, and ask the dentist to discuss the findings and their significance. No finding, diagnosis, or treatment decision is supplied in this case. Technical vocabulary supports communication; it does not qualify a learner to read an image.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which distinction is necessary?

A. Producing an image and interpreting it are different activities.
B. Displaying the image confirms the diagnosis recorded elsewhere.
C. Correctly positioning a receptor establishes authority to recommend treatment.
D. Knowing the term radiolucent establishes what this feature represents.

2. Which question should reach the dentist?

A. Can the assistant invent a reassuring result?
B. Can all uncertainty be deleted from the record?
C. Can an annotation replace the explanation?
D. Does the image support treatment, and how does it relate to the examination and options?

Answers and explanations, page 133

Listen for the distinction

Repeat the transfer with radiopaque and crown. Preserve the question rather than confirming a need for treatment.

Technical confidence should not become a diagnosis

A patient may expect any team member near an image to explain it. Acknowledge the specific question and make the route to an answer clear. Avoid casual reassurance or an alarming guess based on an apparent shadow, color, or shape.

F Say it / transfer to a new case

Route a question about an imaging term

A patient asks whether a note saying radiopaque proves that a tooth needs a crown. The assistant is not authorized to interpret the image. The dentist has not yet explained the note.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

dentist / crown / established / radiopaque

01 **Assistant:** The term in your question is [1] _____.

02 **Patient:** I want to know whether I need a [2] _____.

03 **Assistant:** The explanation needs the [3] _____.

04 **Assistant:** No treatment conclusion has been [4] _____ here.

CHECK THE GAPS

1. radiopaque / **2.** crown / **3.** dentist / **4.** established

Why these answers fit, page 133

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Explain the route to an answer, not the image". Marco: "I appreciate that. I do not want reassurance if it is not justified, but I also do not want a frightening answer based on a guess." Suki: [missing response] Marco: "If a treatment is recommended, I want to understand why that treatment follows from the evidence rather than think the picture makes the decision automatically." Which response belongs in the gap?

A. The clinician can clarify that image annotation and what it refers to. I should not guess the purpose of a mark or present it as proof of a particular condition.

B. That is a question about correlation with other clinical information. I will pass it on along with your question about whether a treatment recommendation is supported and what alternatives exist.

C. The authorized interpreter can explain what is established and what remains uncertain. I can make sure your concern is heard without telling you that everything is normal or definitely abnormal.

D. I will preserve them for the image interpretation discussion. We have identified the questions, not a diagnosis or an agreed procedure, and I will help you obtain the clinical explanation.

Answers and explanations, page 134

Review 2 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Chairside requests and read-backs" and "Imaging questions and authorization". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: A tray used to organize the relevant dental instruments. Which term fits?

A. distinguishing detail
B. explorer
C. handpiece
D. instrument tray

2. A colleague needs the term for this meaning: Cone-beam computed tomography; three-dimensional X-ray imaging used for appropriate indications. Which term fits?

A. diagnostic significance
B. CBCT
C. artifact
D. radiograph

3. Rebuild the exchange from "The pack is present; readiness is not verified". Tessa: "If we locate the original record, we also need to know that it belongs to P18 rather than a similar pack from another processing load." Bruno: [missing response] Tessa: "I can give you the identifiers shown without altering anything. I do not want to copy a number from another pack into the missing record." Which response belongs in the gap?

A. Preserve the actual load number and follow the documentation process. A guessed identifier would hide the problem rather than establish that the required processing and checks occurred.
B. Keep the hold visible. I am checking the replacement now; please tell the dentist that setup is still waiting for verification, not ready yet.
C. Correct. Traceability connects the item to the relevant history. A complete-looking record for a different load would not answer the question about this pack.
D. P18, understood. Has that instrument pack been opened or used? Tell me where it is now so nobody picks it up while I check the record.

Review 2 / Meaning and context / continued

4. Rebuild the exchange from "Explain the route to an answer, not the image". Marco: "Would an earlier image matter? Has the dentist compared this with my previous image?" Suki: [missing response] Marco: "Please ask what the patch means and whether it matches the examination. That is what I need to understand first." Which response belongs in the gap?

A. You can ask whether a comparison image was relevant and reviewed. I will not say that comparison has happened unless it is confirmed in the actual explanation.

B. That is a question about correlation with other clinical information. I will pass it on along with your question about whether a treatment recommendation is supported and what alternatives exist.

C. I will ask the dentist to explain the treatment indication in ordinary language. An image on a screen is not a substitute for the discussion you need before deciding.

D. Displaying a radiograph and interpreting it are different tasks. I can help communicate the question without giving you a diagnosis from something I see on the display.

Answers and explanations, page 158

A Read the situation

LESSON 05 / PATIENT COMFORT, QUESTIONS, AND PAUSES

Acknowledge dental anxiety, communicate a comfort concern, and request an agreed pause signal without guaranteeing sensations.

Reassure without promising a painless procedure

Patient Ava tells assistant Ben she is anxious and asks him to promise that the planned procedure will not hurt. The dentist has not yet discussed comfort measures or her concerns. Ben can listen, relay the concern, and ask the dentist to explain the plan and an appropriate pause signal before proceeding. He cannot guarantee a painless experience, select medication, or invent a sedation arrangement. No specific treatment or anesthetic plan is supplied. The conversation should help Ava ask questions and understand how concerns will be addressed within the actual care process.

People: Ava, Patient; Ben, Dental assistant.

Read the record

Choose one answer per question using the facts above.

1. What does Ava ask Ben to promise?

A. That the laboratory will deliver early
B. That the visit will be free
C. That a particular sedative is prescribed
D. That the procedure will not hurt

2. What has not happened yet?

A. The dentist's discussion of comfort measures and Ava's concerns
B. Any expression of anxiety
C. Ava's arrival
D. Ben hearing the request

3. What can Ben appropriately arrange?

A. An instruction to remain silent during discomfort
B. Communication of the concern and a dentist explanation of the plan and pause signal
C. His own sedation prescription
D. A guarantee of no sensation

Answers and explanations, page 135

B Vocabulary in use

CORE TERMS

dental anxiety
Apprehension or fear related to dental care.
acknowledge dental anxiety

comfort measure
An appropriate action intended to support comfort during care.
discuss comfort measures

sensation
Something physically felt, such as pressure or discomfort.
describe a reported sensation

numbness
Reduced or absent sensation in a particular area.
report numbness accurately

topical anesthetic
An anesthetic applied to a surface for an appropriate clinical purpose.
clarify a topical anesthetic question

analgesia
Relief of pain through the appropriate method or treatment.
clarify an analgesia question

distress
Significant discomfort or emotional upset.
notice reported distress

pain concern
A question or fear about pain that needs to be heard and addressed.
relay a pain concern

pressure
A physical sensation whose expected meaning requires the relevant clinical explanation.
clarify expected pressure

local anesthesia
A clinical method of reducing sensation in a defined area.
discuss local anesthesia

sedation
Medication-related reduction of awareness or anxiety at a defined level under clinical supervision.
discuss sedation options

contraindication
A reason a treatment may be inappropriate in a particular situation.
refer a contraindication question

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

dental anxiety DENTAL ANXIETY

acknowledge dental anxiety

Keep the meaning precise: Apprehension or fear related to dental care.

pressure PRESSURE

clarify expected pressure

Keep the meaning precise: A physical sensation whose expected meaning requires the relevant clinical explanation.

B Precision and professional shorthand

EXTEND YOUR RANGE

medical history
Relevant past and current health information.
update the medical history

previous experience
An earlier event affecting expectations or concerns.
acknowledge a previous experience

trigger
Something associated with an anxiety or distress response.
ask about a reported trigger

pause signal
An agreed way to request a pause during care.
agree a pause signal

ongoing consent
Consent addressed throughout the care process, not only at its beginning.
check ongoing consent

stop request
A request that the activity be stopped through the appropriate safe response.
acknowledge a stop request

explanation before action
Information given before the relevant step occurs.
request explanation before action

communication preference
The person's preferred way of receiving or giving information.
clarify a communication preference

reassurance
Information or support intended to reduce concern without misleading.
give honest reassurance

absolute guarantee
A promise with no stated uncertainty or exceptions.
avoid an unsupported absolute guarantee

clinician review
Assessment or explanation by the responsible qualified clinician.
request clinician review

shared understanding
Agreement about what has been explained and understood.
establish shared understanding

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

contraindication CONTRAINDICATION

refer a contraindication question

Keep the meaning precise: A reason a treatment may be inappropriate in a particular situation.

ongoing consent ONGOING CONSENT

check ongoing consent

Keep the meaning precise: Consent addressed throughout the care process, not only at its beginning.

C Phrases that move the work forward

Acknowledge anxiety

Thank you for telling me that you are worried.

Avoid a guarantee

I cannot promise that you will feel nothing.

Preserve the wording

I will tell the dentist you are worried about pain.

Ask about a signal

Can we agree an appropriate way to request a pause?

Respect a concern

Please tell us if something does not feel as explained.

Clarify preference

Would you prefer an explanation before each stage?

Separate reassurance and certainty

I can help communicate your concern without promising an outcome.

Record accurately

I will record your concern and the explanation actually provided.

Invite the specific concern

What would you like the dentist to know before starting?

Offer a useful next step

I will ask the dentist to discuss the comfort plan with you.

Request explanation

Please explain the likely sensations and available comfort measures.

Confirm the response

How will the team respond if you use that signal?

Avoid minimization

I will not dismiss your concern as just nerves.

Keep clinical choices with the clinician

The dentist needs to discuss any medication or sedation question.

Support questions

You can ask for clarification before deciding whether to continue.

Close the loop

We need to confirm the plan and signal with you, not assume they are understood.

Precision in real exchanges

Purpose: Offer a useful next step
Useful wording: I will ask the dentist to discuss the comfort plan with you.

Purpose: Preserve the wording
Useful wording: I will tell the dentist you are worried about pain.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which reassurance is appropriate?

A. You will definitely feel nothing.

B. Anxiety means your concern is not real.

C. I will choose a sedative for you.

D. I will help you discuss pain concerns and the comfort plan with the dentist before starting.

2. Which statement wrongly treats consent as permanent?

A. You agreed once, so you cannot ask to stop.

B. The clinician should discuss ongoing questions.

C. The pause signal needs explanation.

D. A changed concern should be communicated.

3. A colleague needs the term for this meaning: A question or fear about pain that needs to be heard and addressed. Which term fits?

A. reassurance

B. pain concern

C. topical anesthetic

D. trigger

D Choose the response that fits / continued

4. Rebuild the exchange from "Reassure without promising a painless procedure". Ava: "Last time I did not know whether I was supposed to feel pressure or pain. I could not tell what the person expected me to report." Ben: [missing response] Ava: "I would like to know what can be done to help me feel comfortable. I do not know which choices are appropriate for me." Which response belongs in the gap?

A. Thank you for telling me about your dental anxiety. I cannot promise a painless experience, but I can make sure the dentist discusses your concern and the comfort plan before starting.

B. The clinician should address ongoing consent and respond to questions or a request to stop through the appropriate safe process. An earlier yes does not mean your later concern should be ignored.

C. We can ask about the expected sensation in this procedure. I should not tell you which feeling is acceptable without the clinician's explanation or use the word pressure to dismiss what you report.

D. We should ask the dentist to agree an appropriate pause signal with you before proceeding. The signal and the team's response need to be understood, rather than left for you to guess.

Answers and explanations, page 135

One useful distinction

Pain, pressure, numbness, and anxiety are different words, but the assistant must not use that distinction to dismiss a patient report. The clinician should explain the actual comfort plan and expected sensations; the patient should not be promised an unqualified outcome.

E Conversation

Reassure without promising a painless procedure

Reassure without promising a painless procedure

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

dental anxiety / Sedation / absolute guarantee / sensation / comfort measure / reassurance / pause signal / communication preference / clinician review / ongoing consent

01 **Ava:** Ben, I barely slept. Last time I felt something sharp and could not get anyone's attention. Can you promise this will not hurt?

02 **Ben:** Thank you for telling me about your [1] _____. I cannot promise a painless experience, but I can make sure the dentist discusses your concern and the comfort plan before starting.

03 **Ava:** People say not to worry, but that makes me feel I should stay quiet if something bothers me.

04 **Ben:** I do not want [2] _____ to silence you. Your concern needs a clear response, not a phrase that makes you feel responsible for hiding discomfort.

05 **Ava:** Last time I did not know whether I was supposed to feel pressure or pain. I could not tell what the person expected me to report.

06 **Ben:** We can ask about the expected [3] _____ in this procedure. I should not tell you which feeling is acceptable without the clinician's explanation or use the word pressure to dismiss what you report.

E Conversation / continued

07 **Ava:** I would like to know what can be done to help me feel comfortable. I do not know which choices are appropriate for me.

08 **Ben:** That needs discussion of the actual [4] _____ or measures with the dentist. I will not invent a medication plan, but I will make sure the question is included.

09 **Ava:** Someone mentioned sedation to me, although I do not know whether they meant the same thing as having part of the mouth numb.

10 **Ben:** [5] _____ and local anesthesia are different terms. The dentist can explain their meanings and relevance to you; neither should be assumed simply because a patient has heard the word.

Conversation continues on the next page.

Reassure without promising a painless procedure

CONVERSATION / CONTINUED

11 **Ava:** Please tell them about my previous experience too. I was not sure how to ask for a break once the person had started working.

12 **Ben:** We should ask the dentist to agree an appropriate [6] _____ with you before proceeding. The signal and the team's response need to be understood, rather than left for you to guess.

13 **Ava:** I would also like an explanation before each stage. Surprises make me tense, even when the action itself might not be difficult.

14 **Ben:** I can pass on that [7] _____. The team can discuss how to explain the sequence clearly and check what information helps you feel prepared.

15 **Ava:** If I agree to continue after the explanation and then change my mind, I want to know that I can still say something.

16 **Ben:** The clinician should address [8] _____ and respond to questions or a request to stop through the appropriate safe process. An earlier yes does not mean your later concern should be ignored.

17 **Ava:** That is more useful than a promise that nothing will hurt. I want an honest explanation and a way to communicate, not certainty nobody can give.

18 **Ben:** An unsupported [9] _____ would not help you make an informed choice. I can support a specific conversation about your concern, the plan, and how to ask for attention during care.

19 **Ava:** Could we discuss this before anything starts? I would feel less trapped with an agreed pause signal.

20 **Ben:** I will request that [10] _____ before we treat those questions as resolved. I will record what you asked and what is actually explained and agreed, not an assumed painless outcome.

CHECK THE GAPS

1. dental anxiety / **2.** reassurance / **3.** sensation / **4.** comfort measure / **5.** Sedation / **6.** pause signal / **7.** communication preference / **8.** ongoing consent / **9.** absolute guarantee / **10.** clinician review

Why these answers fit, page 136

F Check accuracy and scope

Reassure without promising a painless procedure

Return to the case

Patient Ava tells assistant Ben she is anxious and asks him to promise that the planned procedure will not hurt. The dentist has not yet discussed comfort measures or her concerns. Ben can listen, relay the concern, and ask the dentist to explain the plan and an appropriate pause signal before proceeding. He cannot guarantee a painless experience, select medication, or invent a sedation arrangement. No specific treatment or anesthetic plan is supplied. The conversation should help Ava ask questions and understand how concerns will be addressed within the actual care process.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What should be confirmed about a pause signal?

A. That it replaces all clinical discussion
B. The actual signal and how the team will respond safely
C. A universal gesture invented by the assistant
D. That the patient must never use it

2. What should the record distinguish?

A. Silence as proof that every question was answered
B. Reported concern, explanation given, and any agreement actually reached
C. A hoped-for painless outcome as a guaranteed fact
D. A requested sedative as an administered drug

Answers and explanations, page 138

Listen for the distinction

Complete and read the transfer using its supplied concern, without dismissing pain as expected pressure.

Reassurance should be honest and specific

Phrases such as you will not feel a thing can sound kind but promise more than the speaker knows. Acknowledge the concern without minimizing it. Explain the next conversation and let the clinician discuss likely sensations, options, and the way to request a pause.

F Say it / transfer to a new case

Communicate a concern about unexpected sensations

A patient requests explanation before each stage and asks how to request a pause. The clinician has not yet confirmed a signal. The assistant cannot guarantee particular sensations.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

each stage / confirmed / pause signal / guarantee

01 **Assistant:** The requested communication preference is explanation before [1] _____.

02 **Patient:** I would like to agree a [2] _____.

03 **Assistant:** The signal is not yet [3] _____.

04 **Assistant:** I cannot provide an absolute [4] _____ about sensations.

CHECK THE GAPS

1. each stage / **2.** pause signal / **3.** confirmed / **4.** guarantee

Why these answers fit, page 138

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Reassure without promising a painless procedure". Ava: "I would also like an explanation before each stage. Surprises make me tense, even when the action itself might not be difficult." Ben: [missing response] Ava: "If I agree to continue after the explanation and then change my mind, I want to know that I can still say something." Which response belongs in the gap?

A. I do not want reassurance to silence you. Your concern needs a clear response, not a phrase that makes you feel responsible for hiding discomfort.

B. Sedation and local anesthesia are different terms. The dentist can explain their meanings and relevance to you; neither should be assumed simply because a patient has heard the word.

C. That needs discussion of the actual comfort measure or measures with the dentist. I will not invent a medication plan, but I will make sure the question is included.

D. I can pass on that communication preference. The team can discuss how to explain the sequence clearly and check what information helps you feel prepared.

Answers and explanations, page 139

A Read the situation

LESSON 06 / RECORDS, IDENTIFIERS, AND FACTUAL HANDOFFS

Report an identifier mismatch before using a record and hand off the facts without guessing which source is correct.

A shared surname is not a verified match

Assistant Diego is preparing a folder for a patient named Morgan. Two patients share that surname. The appointment list shows identifier ending 462, while the folder label shows 426. Diego has not used the folder or disclosed its contents. Lead assistant Priya can check the records through the approved identification process. Neither number is established as correct by this teaching brief. Diego must report the mismatch, preserve confidentiality, and avoid silently changing a label, merging records, or deciding that two similar numbers are merely a harmless typing error.

People: Diego, Dental assistant; Priya, Lead assistant.

Read the record

Choose one answer per question using the facts above.

1. Which mismatch is supplied?

A. The list ends 462 and the folder label ends 426.
B. Both verified identifiers are identical.
C. The surname is absent from both records.
D. The appointment is confirmed for a different day.

2. Which record is established as correct?

A. The folder automatically
B. The list automatically
C. Whichever was printed most recently without verification
D. Neither

3. What has Diego not done?

A. Contacted a route for clarification in the scenario
B. Used the folder or disclosed its contents
C. Noticed the discrepancy
D. Identified the shared surname

Answers and explanations, page 140

B Vocabulary in use

CORE TERMS

patient identifier
Information used to distinguish a particular patient under the approved process.

verify patient identifiers

record number
An identifier assigned to a particular record.

confirm the record number

name collision
Different people sharing the same or similar name in a system.

recognize a name collision

wrong-patient risk
The possibility of using information or providing care for the wrong person.

address wrong-patient risk

folder label
The identifying information shown on a physical folder.

check the folder label

medical-history form
A record of relevant health information supplied and reviewed under the care process.

verify the medical-history form

demographic detail
Recorded information such as name or date of birth.

verify demographic details

identifier mismatch
A disagreement between relevant identifying information.

report an identifier mismatch

duplicate record
More than one record for the same person, requiring proper verification before consolidation.

investigate a possible duplicate record

chart
A patient's clinical record.

verify the chart

appointment list
The schedule of booked patients and visits.

reconcile the appointment list

odontogram
A diagrammatic dental chart of teeth and relevant recorded findings or treatment.

identify the odontogram

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

patient identifier PATIENT IDENTIFIER

verify patient identifiers

Keep the meaning precise: Information used to distinguish a particular patient under the approved process.

duplicate record DUPLICATE RECORD

investigate a possible duplicate record

Keep the meaning precise: More than one record for the same person, requiring proper verification before consolidation.

B Precision and professional shorthand

EXTEND YOUR RANGE

tooth notation
A system for identifying teeth in records.
confirm the tooth notation system

dentition
The arrangement or set of teeth present.
distinguish dentition references

source record
The original record used for verification or reference.
preserve the source record

traceable correction
An amendment that preserves who changed what and when.
make a traceable correction

access control
Restriction of information access to authorized users and purposes.
follow access controls

unverified match
A possible link between person and record not yet confirmed.
flag an unverified match

laterality
The specified side of the body or structure.
verify laterality

chart entry
An item recorded in the patient chart.
make an accurate chart entry

reconciliation
Comparing records and resolving relevant differences through an approved process.
complete record reconciliation

audit trail
A history of recorded actions or changes.
preserve the audit trail

confidential handoff
Transfer of relevant information through an appropriately protected route.
give a confidential handoff

record merge
Combining records after the required verification and authorization.
authorize a record merge

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

odontogram ODONTOGRAM

identify the odontogram

Keep the meaning precise: A diagrammatic dental chart of teeth and relevant recorded findings or treatment.

source record SOURCE RECORD

preserve the source record

Keep the meaning precise: The original record used for verification or reference.

C Phrases that move the work forward

State the discrepancy
The list and folder show different identifier endings.

Name the shared name
Two patients in the records share this surname.

State the current action
I have not used this folder.

Request verification
Please check the match through the approved identification process.

Keep sources separate
Let us preserve what each source currently shows.

Clarify a record change
Any correction needs the proper verified basis and audit trail.

Separate identification and treatment
A record match does not itself confirm the treatment plan.

Report the outcome accurately
I will record the actual verification result, not the expected result.

Read the values clearly
The list ends four-six-two; the label ends four-two-six.

Avoid choosing a winner
I cannot yet establish which record is correct.

Protect information
I have not disclosed its contents.

Avoid a casual correction
I will not change the label just to make it match.

Do not assume duplication
A shared surname does not prove duplicate records.

Protect the handoff
I will discuss the details through the approved confidential route.

Check notation
Which tooth notation system is used in this record?

Close only when resolved
The match remains unverified until the required checks are complete.

Precision in real exchanges

Purpose: Avoid choosing a winner
Useful wording: I cannot yet establish which record is correct.

Purpose: State the current action
Useful wording: I have not used this folder.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which report is precise?

A. I corrected the label without checking so it looks consistent.

B. The list ends 462, the folder ends 426, and I have not used the folder.

C. The folder is definitely wrong because the list is on screen.

D. Both records must belong to one person.

2. Why not merge the records immediately?

A. Similar digits are proof of identity.

B. Record merges never require verification.

C. A shared surname does not establish that they concern the same person.

D. Every name collision is a duplicate record.

3. A colleague needs the term for this meaning: A disagreement between relevant identifying information. Which term fits?

A. dentition

B. identifier mismatch

C. folder label

D. access control

D Choose the response that fits / continued

4. Rebuild the exchange from "A shared surname is not a verified match". Diego: "Four-six-two and four-two-six look like swapped digits. I nearly trusted the screen over the folder, but I have no evidence that the screen is right." Priya: [missing response] Diego: "I will preserve the two values and the fact that the folder has not been used. That seems more helpful than deciding the cause in advance." Which response belongs in the gap?

A. Keep the source record unchanged while we check. Does the surname identify only one patient in our records, or is there another person with the same name?

B. An identifier mismatch does not tell us which source is right. A plausible explanation still needs verification; the screen does not become authoritative merely because it is easier to read.

C. Good. The audit trail should preserve the discrepancy, review, and actual resolution. We will close the issue on verified information rather than a near match that feels convincing.

D. Correct. An odontogram or a remembered treatment history should not replace the required identity checks. The relevant clinical record must first be linked to the correct person through the process.

Answers and explanations, page 140

> **One useful distinction**
>
> A shared surname does not establish that two records concern the same patient. Similar digits do not prove a typing error, either. Preserve both values and use the full approved identification process; these three-digit endings are teaching references, not sufficient real-world identifiers.

E Conversation

A shared surname is not a verified match

A shared surname is not a verified match

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

identifier mismatch / audit trail / wrong-patient risk / traceable correction / confidential handoff / record merge / patient identifier / name collision / odontogram / source record

01 **Diego:** Priya, I have an identifier mismatch before using this folder. The appointment list ends four-six-two, but the folder label ends four-two-six.

02 **Priya:** Keep the [1] _____ unchanged while we check. Does the surname identify only one patient in our records, or is there another person with the same name?

03 **Diego:** There are two patients named Morgan. I have not used the folder or disclosed its contents, and I cannot establish which identifier belongs with this visit.

04 **Priya:** That is a [2] _____, not proof that the records are duplicates. We need the approved identification process before choosing a chart or changing information.

05 **Diego:** Four-six-two and four-two-six look like swapped digits. I nearly trusted the screen over the folder, but I have no evidence that the screen is right.

06 **Priya:** An [3] _____ does not tell us which source is right. A plausible explanation still needs verification; the screen does not become authoritative merely because it is easier to read.

E Conversation / continued

07 **Diego:** I will preserve the two values and the fact that the folder has not been used. That seems more helpful than deciding the cause in advance.

08 **Priya:** Yes. We should address the [4] _____ through the actual process, while keeping the description factual. We have identified a discrepancy before use, not established that treatment was given to the wrong person.

09 **Diego:** Should I ask the person a leading question with one of the numbers, or follow the usual identification questions without suggesting the answer?

10 **Priya:** Follow our approved [5] _____ checks. These partial numbers alone are not enough, and we should not improvise a shortcut that makes an uncertain match appear confirmed.

Conversation continues on the next page.

A shared surname is not a verified match

CONVERSATION / CONTINUED

11 **Diego:** The folder also contains a dental chart, but I have not used its contents to guess whether it looks like the patient I remember.

12 **Priya:** Correct. An [6] _____ or a remembered treatment history should not replace the required identity checks. The relevant clinical record must first be linked to the correct person through the process.

13 **Diego:** If the review shows that one label is wrong, I should not remove the old information without recording how and why it was changed.

14 **Priya:** A [7] _____ is needed under the record procedure. The result must show the supported amendment, not pretend that the original discrepancy never existed.

15 **Diego:** And if the review reveals two records for one person, that would still be a separate authorized process rather than something I do because the surname matches.

16 **Priya:** Exactly. A [8] _____ needs the proper verification and authority. Joining records prematurely could mix information from different patients and make the problem harder to correct.

17 **Diego:** Can we check the details in the staff area? Both patients' names are visible here, and I do not want this conversation carrying into the waiting room.

18 **Priya:** That keeps this a [9] _____. We need enough information to resolve the issue, without exposing either patient's details to people who do not need them.

19 **Diego:** Until the check is complete, my update will say that the match is unverified, the folder has not been used, and you are coordinating the review.

20 **Priya:** Good. The [10] _____ should preserve the discrepancy, review, and actual resolution. We will close the issue on verified information rather than a near match that feels convincing.

CHECK THE GAPS

1. source record / **2.** name collision / **3.** identifier mismatch / **4.** wrong-patient risk / **5.** patient identifier / **6.** odontogram / **7.** traceable correction / **8.** record merge / **9.** confidential handoff / **10.** audit trail

Why these answers fit, page 142

F Check accuracy and scope

A shared surname is not a verified match

Return to the case

Assistant Diego is preparing a folder for a patient named Morgan. Two patients share that surname. The appointment list shows identifier ending 462, while the folder label shows 426. Diego has not used the folder or disclosed its contents. Lead assistant Priya can check the records through the approved identification process. Neither number is established as correct by this teaching brief. Diego must report the mismatch, preserve confidentiality, and avoid silently changing a label, merging records, or deciding that two similar numbers are merely a harmless typing error.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What should a legitimate correction preserve?

A. The verified basis and appropriate traceable history

B. Only the new value with no history

C. An invented earlier verification

D. Another patient's details as a shortcut

2. What are the three-digit endings in this exercise?

A. A universal minimum identification standard

B. Proof of clinical consent

C. Authorization to disclose the folder

D. Teaching references, not a complete real-world identification method

Answers and explanations, page 143

Listen for the distinction

Complete the transfer with its different identifiers; do not choose a record simply because one value looks familiar.

Speak up before a near match becomes a match

A familiar surname and nearly identical digits can feel convincing during a busy clinic. State the exact mismatch without blaming the patient or a colleague. A neutral verification request is more useful than choosing whichever record looks most familiar.

F Say it / transfer to a new case

Report a second identifier mismatch

A schedule reference ends 815 and a folder reference ends 851. Two patients share a surname. The folder has not been used, and neither value is established as correct.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

815 / not been used / unverified / 851

01 **Assistant:** The schedule reference ends [1] _____.

02 **Lead:** The folder reference ends [2] _____.

03 **Assistant:** The folder has [3] _____.

04 **Lead:** The match remains [4] _____.

CHECK THE GAPS

1. 815 / **2.** 851 / **3.** not been used / **4.** unverified

Why these answers fit, page 143

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "A shared surname is not a verified match". Diego: "If the review shows that one label is wrong, I should not remove the old information without recording how and why it was changed." Priya: [missing response] Diego: "And if the review reveals two records for one person, that would still be a separate authorized process rather than something I do because the surname matches." Which response belongs in the gap?

A. Exactly. A record merge needs the proper verification and authority. Joining records prematurely could mix information from different patients and make the problem harder to correct.

B. A traceable correction is needed under the record procedure. The result must show the supported amendment, not pretend that the original discrepancy never existed.

C. That is a name collision, not proof that the records are duplicates. We need the approved identification process before choosing a chart or changing information.

D. That keeps this a confidential handoff. We need enough information to resolve the issue, without exposing either patient's details to people who do not need them.

Answers and explanations, page 144

Review 3 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Patient comfort, questions, and pauses" and "Records, identifiers, and factual handoffs". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: An appropriate action intended to support comfort during care. Which term fits?

A. previous experience
B. communication preference
C. comfort measure
D. local anesthesia

2. A colleague needs the term for this meaning: A patient's clinical record. Which term fits?

A. tooth notation
B. chart
C. patient identifier
D. traceable correction

3. Rebuild the exchange from "Explain the route to an answer, not the image". Marco: "There is also a mark on the screen. I cannot tell whether it is part of the original image or something someone added to point out an area." Suki: [missing response] Marco: "Could it be a technical issue instead? I am not asking you to decide that, but I would like the dentist to address the possibility." Which response belongs in the gap?

A. The clinician can clarify that image annotation and what it refers to. I should not guess the purpose of a mark or present it as proof of a particular condition.
B. I will preserve them for the image interpretation discussion. We have identified the questions, not a diagnosis or an agreed procedure, and I will help you obtain the clinical explanation.
C. The authorized interpreter can explain what is established and what remains uncertain. I can make sure your concern is heard without telling you that everything is normal or definitely abnormal.
D. That is a question about correlation with other clinical information. I will pass it on along with your question about whether a treatment recommendation is supported and what alternatives exist.

Review 3 / Meaning and context / continued

4. Rebuild the exchange from "A shared surname is not a verified match". Diego: "Can we check the details in the staff area? Both patients' names are visible here, and I do not want this conversation carrying into the waiting room." Priya: [missing response] Diego: "Until the check is complete, my update will say that the match is unverified, the folder has not been used, and you are coordinating the review." Which response belongs in the gap?

A. That is a name collision, not proof that the records are duplicates. We need the approved identification process before choosing a chart or changing information.

B. That keeps this a confidential handoff. We need enough information to resolve the issue, without exposing either patient's details to people who do not need them.

C. Keep the source record unchanged while we check. Does the surname identify only one patient in our records, or is there another person with the same name?

D. Exactly. A record merge needs the proper verification and authority. Joining records prematurely could mix information from different patients and make the problem harder to correct.

Answers and explanations, page 161

A Read the situation

LESSON 07 / LABORATORY CASES AND DELIVERY EXPECTATIONS

Distinguish estimated delivery from clinical readiness and agree a useful laboratory update before confirming the patient plan.

> **Expected by noon is not ready for two**
>
> Laboratory coordinator Felix estimates that case C62 will reach the practice on Tuesday by noon. The patient appointment is at 14:00 that day. Delivery is not guaranteed, and the dentist must confirm readiness before the appointment proceeds. Assistant Rina can obtain a laboratory status update on Monday at 16:00. She needs to separate production status, dispatch, receipt, and clinical readiness. The two-hour interval between estimated delivery and the appointment is not a guarantee of sufficient review time. No final decision to proceed or reschedule has been supplied.

People: Rina, Dental assistant; Felix, Laboratory coordinator.

Read the record

Choose one answer per question using the facts above.

1. What is the laboratory estimate?

A. A completed clinical fit review
B. Tuesday by noon, not guaranteed
C. Monday at 16:00 delivery guaranteed
D. Tuesday at 14:00 treatment confirmed

2. When can the lab provide the next status update?

A. A guaranteed arrival time of 16:00
B. Monday at 16:00
C. Tuesday at 14:00 only
D. After the procedure is complete

3. What else is required before proceeding?

A. A two-hour gap alone
B. Dentist confirmation of readiness
C. Only the existence of tracking
D. Automatic consent from the courier

Answers and explanations, page 145

B Vocabulary in use

CORE TERMS

laboratory case
A defined item of work handled by a dental laboratory.

track the laboratory case

case reference
The identifier used to distinguish a particular lab case.

verify the case reference

laboratory prescription
Authorized clinical instructions for the laboratory work.

verify the laboratory prescription

work authorization
Approval for the specified laboratory task.

confirm work authorization

impression
A negative reproduction used for making a model of oral structures.

identify the submitted impression

digital scan
A digital representation used within the relevant laboratory workflow.

verify the submitted digital scan

working model
A model used in producing the specified dental work.

identify the working model

shade
The specified color characteristics of a restoration or appliance.

confirm the shade reference

crown
A restoration covering the relevant tooth structure as clinically prescribed.

track a crown case

bridge
A restoration replacing missing teeth and supported as clinically designed.

identify the bridge case

denture
A prosthesis replacing missing teeth and related structures as prescribed.

identify the denture case

appliance
A device prescribed for a particular dental or orthodontic purpose.

verify the appliance type

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

laboratory case LABORATORY CASE

track the laboratory case

Keep the meaning precise: A defined item of work handled by a dental laboratory.

digital scan DIGITAL SCAN

verify the submitted digital scan

Keep the meaning precise: A digital representation used within the relevant laboratory workflow.

B Precision and professional shorthand

EXTEND YOUR RANGE

try-in

A clinical stage for assessing specified work before final completion or placement.

arrange a try-in appointment

fit assessment

Clinical assessment of whether the item fits as required.

obtain a fit assessment

occlusal check

Clinical assessment relating to how opposing teeth or restorations meet.

refer the occlusal check

remake

Production of replacement work after an appropriate reviewed request.

authorize a remake

production status

The current stage of laboratory manufacture.

request production status

dispatch status

Whether the item has actually been sent.

confirm dispatch status

tracking event

A recorded carrier event, not necessarily final receipt.

interpret a tracking event

estimated delivery

A forecast of when the item may arrive.

qualify estimated delivery

receipt confirmation

Verification that the specified item has arrived at the destination.

obtain receipt confirmation

clinical readiness

Confirmation that the relevant clinical requirements for proceeding are met.

verify clinical readiness

contingency booking

An alternative appointment arrangement considered under the practice process.

discuss a contingency booking

status checkpoint

An agreed time for reviewing and communicating progress.

set a status checkpoint

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

appliance APPLIANCE

verify the appliance type

Keep the meaning precise: A device prescribed for a particular dental or orthodontic purpose.

production status PRODUCTION STATUS

request production status

Keep the meaning precise: The current stage of laboratory manufacture.

C Phrases that move the work forward

Identify the case

I am checking case C62 for Tuesday's appointment.

Repeat the estimate

You estimate arrival Tuesday by noon, without a guarantee.

Name the dependency

The dentist must confirm readiness before the appointment proceeds.

Verify the shipment

Please confirm the case reference associated with the tracking.

Ask about a change

Please tell us if the estimate or production status changes.

Clarify a lab question

Any change to the clinical specification needs the proper authorization.

Request a useful update

At the checkpoint, please distinguish finished, dispatched, and expected arrival.

Confirm the handoff

I will pass the source and limits of the estimate to the dentist.

Request the stage

What is the current production and dispatch status?

Separate the checkpoint

Monday at sixteen hundred is the status update, not delivery.

Avoid a timing assumption

The two-hour gap does not itself establish enough review time.

Distinguish dispatch and receipt

A dispatch message does not confirm arrival at the practice.

Avoid treatment promises

I cannot tell the patient the appointment is ready based only on the estimate.

Keep the patient plan separate

The practice will decide the appointment response using the confirmed information.

Preserve uncertainty

We have an estimate, not receipt confirmation.

Close the status

C62 remains subject to delivery and the required readiness confirmation.

Precision in real exchanges

Purpose: Separate the checkpoint
Useful wording: Monday at sixteen hundred is the status update, not delivery.

Purpose: Name the dependency
Useful wording: The dentist must confirm readiness before the appointment proceeds.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which patient update is accurate?

A. The courier will decide clinical suitability.

B. The lab estimates Tuesday noon delivery; the practice still needs receipt and dentist readiness confirmation.

C. Tuesday treatment is guaranteed because a shipment is expected.

D. Monday 16:00 is the confirmed delivery.

2. What is the interval from noon to 14:00?

A. Two days

B. A guaranteed clinical clearance period

C. Two hours, with no proof that this is sufficient review time

D. Four hours and automatic readiness

3. A colleague needs the term for this meaning: Approval for the specified laboratory task. Which term fits?

A. work authorization

B. crown

C. receipt confirmation

D. occlusal check

D Choose the response that fits / continued

4. Rebuild the exchange from "Expected by noon is not ready for two". Rina: "What can you tell us at Monday's four-o'clock update? It would help to separate what is finished from what has actually left the laboratory." Felix: [missing response] Rina: "If it has been sent, we also need the correct reference and tracking details. We sometimes receive more than one case in the same delivery." Which response belongs in the gap?

A. We should verify the case reference against the shipment. Tracking information for another item would not answer your question about C62, even if the destination is the same.

B. That is the status checkpoint. At that time we will distinguish production, dispatch, and expected arrival. If information changes earlier, we will communicate the change through the agreed route.

C. We can report the production status and whether dispatch has occurred, along with the current delivery estimate. I will not call an item shipped simply because production is expected to finish.

D. Any change to the laboratory prescription needs the appropriate authorization and documentation. We can communicate a question, but the convenience of a delivery date does not decide clinical specifications.

Answers and explanations, page 145

> **One useful distinction**
>
> Produced, dispatched, delivered, checked, and ready for the appointment are different statuses. A tracking event does not necessarily confirm receipt of the correct case, and receipt does not replace the clinical readiness decision by the dentist.

E Conversation

Expected by noon is not ready for two

Expected by noon is not ready for two

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

Receipt confirmation / laboratory prescription / case reference / tracking event / dispatch status / production status / status checkpoint / contingency booking / estimated delivery / clinical readiness

01 **Rina:** Felix, I am checking case C62. The patient is booked for Tuesday at two, and I need to know exactly what your delivery estimate means.

02 **Felix:** The current [1] _____ is Tuesday by noon, but it is not guaranteed. Please keep that qualification attached when you pass the timing to the practice.

03 **Rina:** That leaves two hours on paper. I am worried reception will hear Tuesday noon and tell the patient the two-o'clock fitting is definitely going ahead.

04 **Felix:** Correct. The [2] _____ decision belongs with the dentist after the relevant checks. The laboratory estimate concerns arrival, not whether the appointment can definitely proceed.

05 **Rina:** What can you tell us at Monday's four-o'clock update? It would help to separate what is finished from what has actually left the laboratory.

06 **Felix:** We can report the [3] _____ and whether dispatch has occurred, along with the current delivery estimate. I will not call an item shipped simply because production is expected to finish.

E Conversation / continued

07 **Rina:** If it has been sent, we also need the correct reference and tracking details. We sometimes receive more than one case in the same delivery.

08 **Felix:** We should verify the [4] _____ against the shipment. Tracking information for another item would not answer your question about C62, even if the destination is the same.

09 **Rina:** If the carrier website shows movement, I should describe that movement rather than say we have received the case in the practice.

10 **Felix:** Yes. A [5] _____ can show collection or transit without establishing receipt. Use the actual event and time instead of translating every update into arrived.

Conversation continues on the next page.

Expected by noon is not ready for two

CONVERSATION / CONTINUED

11 **Rina:** And when a package reaches us, the practice needs to confirm the right case was received and follow the relevant checks before calling it ready.

12 **Felix:** Exactly. [6] _____ and clinical readiness are separate. We should keep the sequence clear so a logistics message does not accidentally become a treatment promise.

13 **Rina:** If something changes in the clinical specification, I cannot simply amend it over this call to make the production schedule easier.

14 **Felix:** Any change to the [7] _____ needs the appropriate authorization and documentation. We can communicate a question, but the convenience of a delivery date does not decide clinical specifications.

15 **Rina:** The patient may need time to plan travel. I will ask the practice how to communicate the uncertainty and any alternative appointment arrangements.

16 **Felix:** A [8] _____ may be a practice decision, but I cannot make it from the laboratory. I can provide the clearest available facts and flag changes promptly.

17 **Rina:** Let me read those back: update Monday at four; estimated arrival Tuesday by noon. I will put them in separate fields so nobody confuses the dates.

18 **Felix:** That is the [9] _____. At that time we will distinguish production, dispatch, and expected arrival. If information changes earlier, we will communicate the change through the agreed route.

19 **Rina:** I will tell the dentist that no final decision to proceed or reschedule is established by this conversation. The appointment still depends on actual receipt and readiness.

20 **Felix:** Agreed. Our [10] _____ update will be factual and case-specific. C62 is expected Tuesday by noon, not guaranteed, and the dentist must make the relevant readiness decision.

CHECK THE GAPS

1. estimated delivery / **2.** clinical readiness / **3.** production status / **4.** case reference / **5.** tracking event / **6.** Receipt confirmation / **7.** laboratory prescription / **8.** contingency booking / **9.** status checkpoint / **10.** dispatch status

Why these answers fit, page 147

F Check accuracy and scope

Expected by noon is not ready for two

Return to the case

Laboratory coordinator Felix estimates that case C62 will reach the practice on Tuesday by noon. The patient appointment is at 14:00 that day. Delivery is not guaranteed, and the dentist must confirm readiness before the appointment proceeds. Assistant Rina can obtain a laboratory status update on Monday at 16:00. She needs to separate production status, dispatch, receipt, and clinical readiness. The two-hour interval between estimated delivery and the appointment is not a guarantee of sufficient review time. No final decision to proceed or reschedule has been supplied.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which request is most useful at Monday's checkpoint?

A. A silent change to the laboratory prescription
B. Current production, actual dispatch, revised estimate, and any change affecting delivery
C. A repeat of an old promise regardless of current facts
D. A clinical diagnosis from the courier

2. What does a delivered status not establish by itself?

A. A reason to check the case reference
B. Clinical readiness to proceed
C. That a carrier recorded a delivery event
D. A need to verify receipt details

Answers and explanations, page 148

Listen for the distinction

Use the Wednesday transfer facts below to practice a different checkpoint and delivery estimate.

An estimate is useful when its limits travel with it

Teams may shorten an expected delivery into it will be ready. Preserve the source, time, and conditions when passing information to the practice. A precise update request helps the patient plan without turning laboratory progress into a clinical promise.

F Say it / transfer to a new case

Separate a Wednesday delivery estimate from an update

Case D14 is estimated for Wednesday at 10:00, without a guarantee. The appointment is at 13:00. The lab will update Tuesday at 15:00, and the dentist must still confirm readiness.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

clinical readiness / Tuesday at 15:00 / D14 / Wednesday at 10:00

01 **Assistant:** The case reference is [1] _____.

02 **Lab:** Estimated arrival is [2] _____.

03 **Assistant:** The status update is due [3] _____.

04 **Lab:** The dentist must confirm [4] _____.

CHECK THE GAPS

1. D14 / **2.** Wednesday at 10:00 / **3.** Tuesday at 15:00 / **4.** clinical readiness

Why these answers fit, page 148

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Expected by noon is not ready for two". Rina: "If something changes in the clinical specification, I cannot simply amend it over this call to make the production schedule easier." Felix: [missing response] Rina: "The patient may need time to plan travel. I will ask the practice how to communicate the uncertainty and any alternative appointment arrangements." Which response belongs in the gap?

A. That is the status checkpoint. At that time we will distinguish production, dispatch, and expected arrival. If information changes earlier, we will communicate the change through the agreed route.

B. We should verify the case reference against the shipment. Tracking information for another item would not answer your question about C62, even if the destination is the same.

C. We can report the production status and whether dispatch has occurred, along with the current delivery estimate. I will not call an item shipped simply because production is expected to finish.

D. Any change to the laboratory prescription needs the appropriate authorization and documentation. We can communicate a question, but the convenience of a delivery date does not decide clinical specifications.

Answers and explanations, page 149

A Read the situation

LESSON 08 / AFTERCARE QUESTIONS AND FOLLOW-UP CALLS

Capture a postoperative question accurately, route it to the clinician, and distinguish routine clarification from urgent concerns.

> **An unclear instruction needs an exact clinical answer**
>
> Patient Owen calls assistant Farah because a phrase in his written postoperative instructions is unclear. He quotes "resume your usual routine as directed" and asks what that means for him. Farah is not authorized to reinterpret or change the instructions. The clinician can answer through the practice callback process, but no callback time or clinical answer is yet confirmed. Farah should preserve the exact wording and question, verify the contact through the approved process, and route it appropriately. An urgent or emergency concern must use the designated route rather than an ordinary queue.

People: Owen, Patient; Farah, Dental assistant.

Read the record

Choose one answer per question using the facts above.

1. What does Owen need?

A. Clarification of a phrase in his actual postoperative instructions
B. A new prescription invented by Farah
C. Confirmation that every symptom is normal
D. A laboratory dispatch update

2. What is outside Farah's supplied role?

A. Explaining that timing is not confirmed
B. Reinterpreting or changing the instructions
C. Recording the exact question
D. Using the approved callback process

3. What overrides an ordinary callback queue?

A. A wish to keep every call short
B. The existence of a printed sheet alone
C. An unconfirmed assumption that all callers are stable
D. The appropriate route for an urgent or emergency concern

Answers and explanations, page 150

B Vocabulary in use

CORE TERMS

postoperative
Relating to the period after an operation or procedure.

clarify postoperative instructions

aftercare
Care and instructions relevant after treatment.

discuss an aftercare question

discharge instructions
The information given for care and follow-up after leaving the service.

verify discharge instructions

written direction
An instruction recorded in text whose actual wording matters.

quote the written direction

clinical clarification
An explanation from the appropriate clinician about the intended instruction.

request clinical clarification

verbatim
Using the actual words rather than a changed summary.

record the question verbatim

patient-reported concern
An issue described by the patient and attributed to them.

document a patient-reported concern

callback request
A request for a return call, not confirmation that the call occurred.

log a callback request

contact verification
Checking relevant contact details through the approved process.

complete contact verification

telephone note
A factual record of the call and actions taken.

complete a telephone note

triage route
The designated process for assessing and directing the concern.

use the triage route

urgent concern
A concern requiring timely action under the applicable clinical process.

escalate an urgent concern

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

postoperative POSTOPERATIVE

clarify postoperative instructions

Keep the meaning precise: Relating to the period after an operation or procedure.

verbatim VERBATIM

record the question verbatim

Keep the meaning precise: Using the actual words rather than a changed summary.

B Precision and professional shorthand

EXTEND YOUR RANGE

emergency response

The immediate appropriate response to an emergency under local arrangements.

use the emergency response

review interval

The specified period before reassessment or follow-up.

confirm the review interval

extraction site

The area where a tooth has been removed.

identify an extraction-site question

swelling

Enlargement of tissue whose meaning depends on clinical context.

report swelling accurately

infection concern

A reported worry about possible infection, not itself a diagnosis.

route an infection concern

safety-net advice

Clinician-approved information on what to do if symptoms change or concerns arise.

clarify safety-net advice

follow-up appointment

A later visit arranged for a stated clinical purpose.

verify the follow-up appointment

suture

Material used to hold tissues together as part of a clinical procedure.

clarify a suture question

bleeding

Loss of blood whose severity and context require appropriate assessment.

report bleeding without minimizing it

dry socket

A painful post-extraction complication requiring dental assessment; not a label for any discomfort.

refer a dry-socket question

B Precision and professional shorthand / continued

instruction amendment

An authorized change to the actual clinical directions.

document an instruction amendment

unresolved question

A question for which an appropriate answer has not yet been provided.

track an unresolved question

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

urgent concern URGENT CONCERN

escalate an urgent concern

Keep the meaning precise: A concern requiring timely action under the applicable clinical process.

extraction site EXTRACTION SITE

identify an extraction-site question

Keep the meaning precise: The area where a tooth has been removed.

C Phrases that move the work forward

Welcome the question

Thank you for calling to check the instruction.

Ask for exact wording

Could you read the phrase exactly as it appears?

Preserve the source

I will record that as wording from your instruction sheet.

Clarify the question

What part would you like the clinician to explain?

State the limit

I am not authorized to reinterpret or change that direction.

Offer the route

I can pass the exact question to the clinician through our callback process.

Avoid invented timing

I do not yet have a confirmed callback time.

Verify contact properly

Let us confirm the contact details through our usual process.

Keep urgency separate

An urgent concern needs the designated route, not the ordinary queue.

Avoid a diagnosis

I cannot identify the cause of a symptom from an unassessed description.

Record an attributed report

I will record what you tell me as your report.

Do not replace the instruction

I will not substitute my own timetable for the clinician's direction.

Check the response

Please ask for clarification if the clinician's answer remains unclear.

Confirm any amendment

A changed instruction needs the authorized clinical response.

Track the open question

The question remains unresolved until the appropriate answer is provided.

Close accurately

I have logged the request; I have not given a new clinical instruction.

Precision in real exchanges

Purpose: Clarify the question
Useful wording: What part would you like the clinician to explain?

Purpose: State the limit
Useful wording: I am not authorized to reinterpret or change that direction.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which response is appropriate?

A. Any written instruction can be replaced by my personal advice.

B. Calling proves no urgent concern exists.

C. Please quote the phrase; I will route the exact question for clinical clarification without changing it.

D. As directed always means immediately.

2. What should Farah say about callback timing?

A. The clinician will call in five minutes without confirmation.

B. The answer has already been given because the request was logged.

C. Every callback may safely wait indefinitely.

D. No time is confirmed yet.

3. A colleague needs the term for this meaning: An instruction recorded in text whose actual wording matters. Which term fits?

A. infection concern

B. written direction

C. review interval

D. contact verification

D Choose the response that fits / continued

4. Rebuild the exchange from "An unclear instruction needs an exact clinical answer". Owen: "I am glad you said that. I do not need a general answer copied from someone else's visit; I need to know what the direction means for mine." Farah: [missing response] Owen: "Will the dentist actually see that wording? Last time I was told a message had been logged, but I still did not know what to do." Which response belongs in the gap?

A. Thank you for checking the written direction. Could you read the phrase exactly as it appears, then tell me which part you want the clinician to explain?

B. A callback request is not an answer. I will keep the response pending until the actual clinical communication is recorded, rather than treat the administrative entry as resolution.

C. Any instruction amendment needs to be communicated and recorded through the proper process. I will not create a competing instruction myself or silently replace the original text.

D. That requires clinical clarification. I can pass the exact wording and your question through the practice callback process, using the relevant verified record and contact details.

Answers and explanations, page 150

> **One useful distinction**
>
> A familiar phrase can still have a patient-specific meaning. Do not silently replace as directed with an invented timetable or permission. Preserve the wording and have the clinician explain the intended direction and any relevant conditions.

E Conversation

An unclear instruction needs an exact clinical answer

An unclear instruction needs an exact clinical answer

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

telephone note / patient-reported concern / contact verification / instruction amendment / written direction / clinical clarification / verbatim / aftercare / urgent concern / callback request

01 **Owen:** Farah, I am home with the instruction sheet, but one sentence has me stuck. Could someone explain it before I guess what I am supposed to do?

02 **Farah:** Thank you for checking the [1] _____. Could you read the phrase exactly as it appears, then tell me which part you want the clinician to explain?

03 **Owen:** It says, resume your usual routine as directed. I do not know what as directed refers to or whether another part of the sheet explains it.

04 **Farah:** I will record that [2] _____, along with your question. I am not authorized to reinterpret or change the instruction, so I should not supply a timetable that the clinician has not confirmed.

05 **Owen:** I am glad you said that. I do not need a general answer copied from someone else's visit; I need to know what the direction means for mine.

06 **Farah:** That requires [3] _____. I can pass the exact wording and your question through the practice callback process, using the relevant verified record and contact details.

E Conversation / continued

07 **Owen:** Will the dentist actually see that wording? Last time I was told a message had been logged, but I still did not know what to do.

08 **Farah:** A [4] _____ is not an answer. I will keep the response pending until the actual clinical communication is recorded, rather than treat the administrative entry as resolution.

09 **Owen:** When should I expect the call? Please tell me if the time is not confirmed.

10 **Farah:** I do not have a confirmed time. We should complete [5] _____ through our process and use the designated route for an update on timing, rather than promise five minutes without confirmation.

Conversation continues on the next page.

An unclear instruction needs an exact clinical answer

CONVERSATION / CONTINUED

11 **Owen:** If I report a new problem as well as the wording question, should that go into the same ordinary queue automatically?

12 **Farah:** No. An [6] _____ must use the designated route, including emergency response when appropriate. A call that begins with a routine question can still need a different response.

13 **Owen:** I understand. I do not want anyone to assume that every symptom after treatment is normal, or to guess a diagnosis from one word.

14 **Farah:** I will preserve any [7] _____ accurately and follow the practice process. I should not reassure you that a described symptom is harmless without the appropriate clinical assessment.

15 **Owen:** When the clinician responds, I want to be able to repeat what I understood and ask again if the explanation is still unclear.

16 **Farah:** That can help clarify the [8] _____ discussion. Ask about anything unclear in the actual response rather than rely on a guess simply because the original phrase sounded familiar.

17 **Owen:** And if the clinician changes the direction, I would like the new wording clear. I do not want two different instructions without knowing which applies.

18 **Farah:** Any [9] _____ needs to be communicated and recorded through the proper process. I will not create a competing instruction myself or silently replace the original text.

An unclear instruction needs an exact clinical answer / continued

19 **Owen:** Please record the phrase and my question about its meaning. You have not given me a new clinical direction.

20 **Farah:** Correct. My [10] _____ will preserve the question, the action taken, and the pending response. We will use the appropriate route for any urgent concern rather than let the callback process delay it.

CHECK THE GAPS

1. written direction / **2.** verbatim / **3.** clinical clarification / **4.** callback request / **5.** contact verification / **6.** urgent concern / **7.** patient-reported concern / **8.** aftercare / **9.** instruction amendment / **10.** telephone note

Why these answers fit, page 152

F Check accuracy and scope

An unclear instruction needs an exact clinical answer

Return to the case

Patient Owen calls assistant Farah because a phrase in his written postoperative instructions is unclear. He quotes "resume your usual routine as directed" and asks what that means for him. Farah is not authorized to reinterpret or change the instructions. The clinician can answer through the practice callback process, but no callback time or clinical answer is yet confirmed. Farah should preserve the exact wording and question, verify the contact through the approved process, and route it appropriately. An urgent or emergency concern must use the designated route rather than an ordinary queue.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What should happen if the caller reports an urgent concern?

A. Diagnose it from a single word.
B. Tell the caller that all postoperative symptoms are normal.
C. Use the designated urgent or emergency route as appropriate.
D. Leave it in the ordinary queue regardless.

2. Which note preserves the distinction?

A. Exact phrase and question logged; clinician response pending
B. New clinical advice supplied by Farah when none was authorized
C. Concern resolved because a form was completed
D. Original instruction silently replaced

Answers and explanations, page 153

Listen for the distinction

Complete the transfer using review as arranged and the missing appointment date, not an invented interval.

Clarification is not a nuisance

Patients may feel embarrassed to call about a phrase that seems simple to staff. Thank the caller for checking. Use exact wording and an accurate handoff so the clinician answers the real question rather than a summary that silently changes the instruction.

F Say it / transfer to a new case

Route an unclear follow-up interval

A patient asks what review as arranged means on the instruction sheet. No appointment date is confirmed in the available record. The assistant cannot invent an interval; the clinician and scheduling team must clarify through the approved process.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

interval / appointment date / review as arranged / unresolved

01 **Assistant:** The quoted phrase is [1] _____.

02 **Patient:** The missing information is the confirmed [2] _____.

03 **Assistant:** I cannot invent a review [3] _____.

04 **Assistant:** The question is still [4] _____.

CHECK THE GAPS

1. review as arranged / **2.** appointment date / **3.** interval / **4.** unresolved

Why these answers fit, page 153

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "An unclear instruction needs an exact clinical answer". Owen: "I understand. I do not want anyone to assume that every symptom after treatment is normal, or to guess a diagnosis from one word." Farah: [missing response] Owen: "When the clinician responds, I want to be able to repeat what I understood and ask again if the explanation is still unclear." Which response belongs in the gap?

A. A callback request is not an answer. I will keep the response pending until the actual clinical communication is recorded, rather than treat the administrative entry as resolution.

B. That can help clarify the aftercare discussion. Ask about anything unclear in the actual response rather than rely on a guess simply because the original phrase sounded familiar.

C. I will preserve any patient-reported concern accurately and follow the practice process. I should not reassure you that a described symptom is harmless without the appropriate clinical assessment.

D. No. An urgent concern must use the designated route, including emergency response when appropriate. A call that begins with a routine question can still need a different response.

Answers and explanations, page 154

Review 4 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Laboratory cases and delivery expectations" and "Aftercare questions and follow-up calls". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Authorized clinical instructions for the laboratory work. Which term fits?

A. shade
B. laboratory prescription
C. fit assessment
D. estimated delivery

2. A colleague needs the term for this meaning: A request for a return call, not confirmation that the call occurred. Which term fits?

A. postoperative
B. callback request
C. emergency response
D. swelling

3. Rebuild the exchange from "A shared surname is not a verified match". Diego: "Should I ask the person a leading question with one of the numbers, or follow the usual identification questions without suggesting the answer?" Priya: [missing response] Diego: "The folder also contains a dental chart, but I have not used its contents to guess whether it looks like the patient I remember." Which response belongs in the gap?

A. Exactly. A record merge needs the proper verification and authority. Joining records prematurely could mix information from different patients and make the problem harder to correct.
B. That keeps this a confidential handoff. We need enough information to resolve the issue, without exposing either patient's details to people who do not need them.
C. Keep the source record unchanged while we check. Does the surname identify only one patient in our records, or is there another person with the same name?
D. Follow our approved patient identifier checks. These partial numbers alone are not enough, and we should not improvise a shortcut that makes an uncertain match appear confirmed.

Review 4 / Meaning and context / continued

4. Rebuild the exchange from "An unclear instruction needs an exact clinical answer". Owen: "And if the clinician changes the direction, I would like the new wording clear. I do not want two different instructions without knowing which applies." Farah: [missing response] Owen: "Please record the phrase and my question about its meaning. You have not given me a new clinical direction." Which response belongs in the gap?

A. Any instruction amendment needs to be communicated and recorded through the proper process. I will not create a competing instruction myself or silently replace the original text.

B. That requires clinical clarification. I can pass the exact wording and your question through the practice callback process, using the relevant verified record and contact details.

C. Thank you for checking the written direction. Could you read the phrase exactly as it appears, then tell me which part you want the clinician to explain?

D. A callback request is not an answer. I will keep the response pending until the actual clinical communication is recorded, rather than treat the administrative entry as resolution.

Answers and explanations, page 164

Language for the next case

Which tooth does twelve mean?

Universal

Universal number twelve identifies the permanent upper-left first premolar; the identical digits in FDI notation identify a different tooth.

Dr. Imani: The permanent upper-left first premolar. In our Universal chart that is number twelve. Please read back the tooth name as well as the number.

FDI

FDI two-four corresponds to the intended permanent upper-left first premolar; FDI one-two is not the same tooth as Universal twelve.

Dr. Imani: Exactly. For the intended first premolar, the FDI equivalent is two-four. The conversion helps us check the language; it does not establish what an unlabeled source intended.

notation system

The notation system must accompany the tooth designation so a reader knows how to interpret the digits.

Dr. Imani: Correct. Keep the notation system visible in the appropriate field. A correct conversion attached to the wrong source assumption would still be wrong.

distal

The confirmed MOD entry includes distal as well as mesial and occlusal; recording MO would omit one of the specified surfaces.

Dr. Imani: Yes. Include distal as well as mesial and occlusal. Do not drop the final letter because the imported print is faint.

planned

The dentist explicitly confirms planned status; the dialogue supplies no completed procedure to document or bill as completed.

Dr. Imani: Use planned. This instruction does not say that today's treatment has been carried out. Keep any existing restoration information separate from this proposed work.

audit trail

An audit trail preserves the source wording and the dated, attributable clarification instead of concealing the original ambiguity.

Dr. Imani: Good. If the imported record needs an amendment, preserve the audit trail. Do not make it look as though the original system label was present all along.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

Which tooth does twelve mean?

An imported note says 12 MOD without a notation-system label. The local chart uses Universal numbering. Dentist Imani confirms the intended tooth is the permanent upper-left first premolar, surfaces mesial, occlusal, distal, and the entry is planned, not completed. Assistant Jo verifies the charting instruction.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

Universal / distal / notation system / planned / audit trail / FDI

01 **Jo:** Before I enter this, the imported note says twelve MOD. It does not name the numbering system. Which tooth did you intend?

02 **Dr. Imani:** The permanent upper-left first premolar. In our [1] _____ chart that is number twelve. Please read back the tooth name as well as the number.

03 **Jo:** Permanent upper-left first premolar, Universal twelve. I nearly read the imported twelve as FDI one-two, which would be the upper-right lateral incisor.

04 **Dr. Imani:** Exactly. For the intended first premolar, the [2] _____ equivalent is two-four. The conversion helps us check the language; it does not establish what an unlabeled source intended.

05 **Jo:** So I should preserve the imported wording and record your clarification, not silently replace every twelve in the source with twenty-four.

06 **Dr. Imani:** Correct. Keep the [3] _____ visible in the appropriate field. A correct conversion attached to the wrong source assumption would still be wrong.

Conversation / continued

07 **Jo:** Now the surfaces: I read MOD as mesial, occlusal, and distal. Is that the full entry, rather than MO?

08 **Dr. Imani:** Yes. Include [4] _____ as well as mesial and occlusal. Do not drop the final letter because the imported print is faint.

09 **Jo:** The screen offers existing, planned, and completed. Which status belongs with this entry? I do not want a treatment plan recorded as work already performed.

10 **Dr. Imani:** Use [5] _____. This instruction does not say that today's treatment has been carried out. Keep any existing restoration information separate from this proposed work.

Conversation continues on the next page.

Which tooth does twelve mean?

CONVERSATION / CONTINUED

11 **Jo:** I have the three parts: Universal twelve, MOD, planned. Does the charting clarification itself confirm consent to proceed today?

12 **Dr. Imani:** No. Confirming a record entry is not the whole clinical and consent process. We still follow those processes for the actual patient and proposed care.

13 **Jo:** I will not create a completed procedure entry or a charge from this clarification. I will enter only what you have actually confirmed within my authorized role.

14 **Dr. Imani:** Good. If the imported record needs an amendment, preserve the [6] _____. Do not make it look as though the original system label was present all along.

15 **Jo:** There is another twelve on the next page, but it has no tooth name. Should I apply today's clarification to that one too?

16 **Dr. Imani:** Not automatically. It may refer to another entry or a different context. Flag it separately so its source and intended tooth can be checked.

17 **Jo:** Understood. Today's confirmation covers this entry only. I will leave the second ambiguity visible rather than make the whole record appear reconciled.

18 **Dr. Imani:** That is right. Also keep left and right from the patient's perspective. A display facing us does not reverse the anatomical tooth name.

19 **Jo:** Final read-back: permanent upper-left first premolar, Universal twelve, equivalent FDI two-four; mesial, occlusal, distal; planned. Separate unlabeled entry still needs clarification.

20 **Dr. Imani:** Confirmed. Record this clarification with its source and date through our process. The other entry remains unresolved, and no completed treatment is established by this exchange.

CHECK THE GAPS

1. Universal / **2.** FDI / **3.** notation system / **4.** distal / **5.** planned / **6.** audit trail

Why these answers fit, page 167

Separate the number from the status

The dentist confirms the permanent upper-right lateral incisor: FDI 12, Universal 7. The entry describes an existing restoration, not a newly completed procedure. The imported notation must remain traceable; the assistant is authorized to record this clarification.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

12 / 7 / existing / audit trail

01 **Assistant:** In FDI notation, the confirmed tooth is [1] _____ .

02 **Dentist:** Its Universal designation is [2] _____ .

03 **Assistant:** The confirmed record status is [3] _____ .

04 **Dentist:** Preserve the source and clarification in the [4] _____ .

CHECK THE GAPS

1. 12 / **2.** 7 / **3.** existing / **4.** audit trail

Why these answers fit, page 167

Keep the distinction audible

Read back the notation system, anatomical tooth, surfaces, and record status without inventing a treatment decision.

Language for the next case

Update the medical history

no known drug allergies

The existing no-known-allergies entry conflicts with a newly reported possible drug reaction and needs review; it should not be carried forward unquestioned.

Ravi: I remembered a rash after an antibiotic years ago. The form says no known drug allergies, but I am not sure that still describes what I have told you.

drug name

The drug name is unknown in the supplied account; choosing a familiar antibiotic would invent information rather than clarify the history.

Ravi: A past event. I have no rash or other symptoms now. I cannot remember the drug name, though my old pharmacy might have a record.

reported reaction

The reported reaction is Ravi's account of red, itchy patches, not a confirmed allergy diagnosis or a clinician's documented conclusion.

Ravi: Red, itchy patches on my arms. I spoke to a doctor, but I cannot remember what they called it. That is my reported reaction, not a diagnosis I can confirm.

unknown

Unknown timing is a missing fact; it must not be converted into an immediate reaction or a statement that no reaction occurred.

Ravi: No. Please mark the timing as unknown. I do not want to guess a date just to finish the form.

information source

The information source is the patient's recollection until another record is actually obtained and checked; a possible pharmacy record is not yet corroborating evidence.

Ravi: Not with me. I can give the old pharmacy's details. My recollection is the current information source, not something you have already verified with them.

clinical review

Clinical review is needed for assessment and advice; collecting the history does not authorize the assistant to change medicines or clear the patient for treatment.

Mei: I cannot advise you to stop or change them. I will relay that question for clinical review, together with the information you have given me.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

Update the medical history

The chart says no known drug allergies. Patient Ravi now reports a past rash after an antibiotic but cannot recall the drug, year, or number of doses. There is no current reaction. Assistant Mei may collect history, but the dentist must assess the report before the visit proceeds.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

drug name / information source / unknown / reported reaction / clinical review / no known drug allergies

01 **Mei:** Before we continue, has anything changed in your medical history or medicines since the last visit? That includes anything you forgot to mention before.

02 **Ravi:** I remembered a rash after an antibiotic years ago. The form says [1] _____, but I am not sure that still describes what I have told you.

03 **Mei:** Thank you for raising it. Are you describing a past event, or is anything happening now?

04 **Ravi:** A past event. I have no rash or other symptoms now. I cannot remember the [2] _____, though my old pharmacy might have a record.

05 **Mei:** I will record what you recall and what you do not. What did you notice at the time, and did anyone assess it?

06 **Ravi:** Red, itchy patches on my arms. I spoke to a doctor, but I cannot remember what they called it. That is my [3] _____, not a diagnosis I can confirm.

07 **Mei:** Do you remember the year, how long you had taken the medicine, or how soon the patches appeared after a dose?

08 **Ravi:** No. Please mark the timing as [4] _____. I do not want to guess a date just to finish the form.

09 **Mei:** That is helpful. Do you have a letter, medication list, or other record we could ask about through the proper process?

10 **Ravi:** Not with me. I can give the old pharmacy's details. My recollection is the current [5] _____, not something you have already verified with them.

Conversation continues on the next page.

Update the medical history

CONVERSATION / CONTINUED

11 **Mei:** Exactly. I will identify you as the source and keep the gaps visible. I will also tell the dentist that the old entry needs review before we proceed.

12 **Ravi:** Does that mean you are adding penicillin allergy? That is the antibiotic name people usually mention to me.

13 **Mei:** I cannot substitute that name for an unknown medicine or diagnose the cause. The dentist needs to assess your account and decide the appropriate next step.

14 **Ravi:** All right. I also started a supplement last month. Do you only need prescribed medicines for this history update?

15 **Mei:** Please include the medicines and supplements requested by our history process, including nonprescription products. Bring the names or packaging details rather than relying on a description such as a small white tablet.

16 **Ravi:** Should I stop taking my usual tablets until somebody has checked the list?

17 **Mei:** I cannot advise you to stop or change them. I will relay that question for [6] _____, together with the information you have given me.

18 **Ravi:** Please make sure the dentist knows I am asking about a past event, not saying I have a reaction here today.

19 **Mei:** I will. My handoff will separate the past rash, unknown antibiotic and timing, no current reaction reported, old chart entry, and your question about regular medicines.

20 **Ravi:** That is accurate. I can supply the pharmacy details now, but please do not say the history has been confirmed until the proper review has actually happened.

CHECK THE GAPS

1. no known drug allergies / **2.** drug name / **3.** reported reaction / **4.** unknown / **5.** information source / **6.** clinical review

Why these answers fit, page 169

Do not turn uncertainty into an allergy label

A patient reports nausea after a pain medicine two years ago; its name is unknown. No current symptoms are reported. The old chart says no known allergies. The assistant records the account and routes it to the clinician; no allergy diagnosis or advice to change medicines has been given.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

unknown / allergy / nausea / review

01 **Assistant:** The symptom the patient recalls is [1] _____ .

02 **Patient:** The medicine's name is [2] _____ .

03 **Assistant:** This is a reported reaction, not a confirmed [3] _____ .

04 **Patient:** The advice and history update still need clinician [4] _____ .

CHECK THE GAPS

1. nausea / **2.** unknown / **3.** allergy / **4.** review

Why these answers fit, page 169

Keep the distinction audible

Elicit and attribute a reported past reaction, distinguish unknown from none known, and obtain clinical review without diagnosing an allergy.

Language for the next case

Check the material, not the box

usable stock

Only the six verified, in-date syringes count as usable stock; the four expired syringes remain physically present but unavailable for use.

Sam: Separate physical count from usable stock. Put the expired four on the required hold and check the other lot's identity and status before counting it as available.

lot number

The lot number distinguishes C27 from expired C18 even though their product and shade are the same.

Sam: Then usable quantity is six. The lot number matters because two visually similar boxes can have different histories and expiry dates.

six

Twelve target syringes minus six usable syringes leaves a six-syringe replenishment requirement, with no outstanding order to subtract.

Sam: Raise the order under our process to the target of twelve: six additional syringes. Do not order only two; two would reach the trigger, not our stated target.

first-expiry-first-out

First-expiry-first-out prioritizes the earliest expiry among eligible stock; it does not authorize using already expired material.

Sam: Correct. Also use first-expiry-first-out for eligible stock under our procedure. That means comparing expiry dates, not assuming the oldest delivery is always the first to expire.

expiry date

The expiry date is a separate label field from receipt date and lot number; opening or moving a pack does not extend it.

Sam: That is why the expiry date needs its own check. We do not extend it by opening the box or moving the material to a different drawer.

instructions for use

The instructions for use are product-specific; another material's storage, handling, or curing directions cannot be applied merely because the products are used together.

Sam: Exactly. Check each product's current instructions for use. Do not carry over another manufacturer's storage or handling directions because the packaging looks familiar.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

Check the material, not the box

Fictional stock check on 10 October 2026: composite A2 lot C18 has 4 syringes, expiry 30 September; lot C27 has 6, expiry 31 December. Same product and shade. C27's storage and other required checks are verified. The reorder point is 8 usable syringes; target stock is 12, with none on order.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

expiry date / lot number / first-expiry-first-out / usable stock / six / instructions for use

01 **Ella:** The inventory screen shows ten A2 composite syringes, but four belong to a lot that expired on September thirtieth. Today is October tenth.

02 **Sam:** Separate physical count from [1] _____. Put the expired four on the required hold and check the other lot's identity and status before counting it as available.

03 **Ella:** The six are the same product and shade A2, lot C27, expiry December thirty-first. Its storage and other required checks are verified in our record.

04 **Sam:** Then usable quantity is six. The [2] _____ matters because two visually similar boxes can have different histories and expiry dates.

05 **Ella:** The reorder point is eight usable syringes. Six is below it, even though the shelf contains ten. Nothing is already on order.

06 **Sam:** Raise the order under our process to the target of twelve: [3] _____ additional syringes. Do not order only two; two would reach the trigger, not our stated target.

Conversation / continued

07 **Ella:** I will check the exact product, shade, pack size, and unit of purchase. Six syringes is not automatically six boxes.

08 **Sam:** Correct. Also use [4] _____ for eligible stock under our procedure. That means comparing expiry dates, not assuming the oldest delivery is always the first to expire.

09 **Ella:** The expired lot arrived later than C27. If I had followed delivery order alone, I would not have caught that difference.

10 **Sam:** That is why the [5] _____ needs its own check. We do not extend it by opening the box or moving the material to a different drawer.

Conversation continues on the next page.

Check the material, not the box

CONVERSATION / CONTINUED

11 **Ella:** There is also an A3 syringe on the shelf. Can I include it in the A2 available count because it is the same product range?

12 **Sam:** No. A different shade is not the requested item. Any clinical substitution needs the appropriate decision, not an inventory assumption made to avoid a shortage.

13 **Ella:** And the bonding agent is a separate stock item, even though it is used in the same restorative workflow. Its count cannot fill the composite shortage.

14 **Sam:** Exactly. Check each product's current [6] _____. Do not carry over another manufacturer's storage or handling directions because the packaging looks familiar.

15 **Ella:** For the expired four, I will record C18, quantity four, the printed expiry, and the hold location. The stock lead will arrange the authorized disposition.

16 **Sam:** Keep those units out of available inventory while preserving their traceability. A hold is not the same as a completed supplier return or documented disposal.

17 **Ella:** The order request will specify six additional A2 syringes, subject to approval. I will confirm the supplier's pack quantity before sending it.

18 **Sam:** And do not add that request to received stock. Approved, ordered, dispatched, and received quantities are different stages, even when we expect delivery soon.

19 **Ella:** Summary: ten physical, four expired on hold, six usable, six to replenish to twelve. A3 and bonding agent remain separate items.

20 **Sam:** Agreed. Update the counts and the hold record, then route the replenishment request. We have identified the requirement; we have not claimed that new stock has arrived.

CHECK THE GAPS

1. usable stock / **2.** lot number / **3.** six / **4.** first-expiry-first-out / **5.** expiry date / **6.** instructions for use

Why these answers fit, page 171

Count syringes rather than cartons

Fictional target: 15 usable syringes. There are 5 verified usable syringes and 3 expired units on hold; nothing is on order. The supplier sells cartons of 5 syringes. A purchase request is prepared but neither approved nor received.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

10 / received / 8 / 2

01 **Assistant:** Physical quantity is [1] _____ syringes.

02 **Lead:** The replenishment requirement is [2] _____ syringes.

03 **Assistant:** That quantity equals [3] _____ supplier cartons.

04 **Lead:** New stock has not yet been [4] _____ .

CHECK THE GAPS

1. 8 / **2.** 10 / **3.** 2 / **4.** received

Why these answers fit, page 171

Keep the distinction audible

Separate product identity, shade, lot, expiry, and availability when reconciling restorative-material stock.

01 / Answers and explanations

A / Read the situation

A1. What does the schedule show?

A. A thirty-minute consultation

The schedule confirms a consultation and its duration, not a particular procedure.

Return to the exercise, page 4

A2. What does Lena expect?

D. A filling today

Lena's expectation is a filling, which differs from the recorded appointment type.

Return to the exercise, page 4

A3. Who can clarify the clinical plan?

B. The dentist before Lena decides whether to continue

The dentist is available to explain the plan; a booking entry does not supply a clinical decision.

Return to the exercise, page 4

D / Response choices

D1. Which response is accurate?

C. The record shows a consultation; the dentist can clarify the plan before you decide.

The response separates scheduling, clinical explanation, and the patient's decision.

Return to the exercise, page 8

D2. Which statement overpromises?

B. Your insurance will definitely pay because the visit is booked.

A booking does not establish coverage, payment, or the final treatment charge.

Return to the exercise, page 8

01 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: The category of visit recorded in the schedule. Which term fits?

C. appointment type

A Not here. clinical decision: A decision requiring the appropriate clinical assessment and authority. The usual combination is "refer a clinical decision".

B Not here. treatment option: A possible approach requiring appropriate explanation and assessment. The usual combination is "compare treatment options".

C Correct. appointment type: The category of visit recorded in the schedule. The usual combination is "verify the appointment type".

D Not here. procedure code: A coded description of a dental service within a particular system. The usual combination is "verify the procedure code".

Return to the exercise, page 8

D4. Rebuild the exchange from "A consultation is not a confirmed filling". Lena: "Does the shorter appointment mean the dentist has already decided there is nothing wrong? I do not want my concern dismissed before anyone looks." Amir: [missing response] Lena: "My main question is whether this tooth needs a filling at all. I also want to know what the alternatives would involve." Which response belongs in the gap?

B. No. The appointment duration does not establish a diagnosis or the seriousness of your concern. I will tell the dentist what you expected and ask them to clarify the purpose with you.

A Not here. This line belongs to turn 18, after Lena says: "Then I would like to ask the dentist to explain what can be decided today and whether another appointment might be needed." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Amir's response in turn 6. It connects Lena's preceding point with Lena's follow-up. Appointment duration is the time allocated in the schedule, not a clinical judgment about the tooth.

C Not here. This line belongs to turn 2, after Lena says: "I am here for the filling. I took the afternoon off, so I hope we can finish it today rather than arrange another visit." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 8, after Lena says: "My main question is whether this tooth needs a filling at all. I also want to know what the alternatives would involve." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 8

E / Conversation

1. patient expectation

Turn 2. The patient expectation is a filling today, which must be acknowledged without treating it as a confirmed booking.

01 / Answers and explanations / continued

2. appointment type

Turn 4. The appointment type identifies the scheduled consultation and does not automatically authorize or confirm treatment.

3. appointment duration

Turn 6. Appointment duration is the time allocated in the schedule, not a clinical judgment about the tooth.

4. chief concern

Turn 8. The chief concern is the patient's central question about the tooth and treatment options, not an assistant's diagnosis.

5. treatment estimate

Turn 10. The treatment estimate must specify the proposed care and cost basis rather than remain an unexplained number.

6. coverage estimate

Turn 12. A coverage estimate is provisional payment information and is not a guarantee of insurer payment or final patient cost.

7. Informed consent

Turn 14. Informed consent requires the applicable explanation and agreement process; staying for a consultation is not blanket treatment permission.

8. booking discrepancy

Turn 16. The booking discrepancy is the difference between the expected filling and recorded consultation, without assigning blame or demanding treatment.

9. treatment plan

Turn 18. The treatment plan concerns the proposed care and sequence; the dentist must explain it rather than the assistant promise same-day treatment.

10. decision time

Turn 20. Decision time gives the patient an opportunity to understand the explanation and ask questions before agreeing to the proposed next step.

Return to the conversation, page 10

01 / Answers and explanations / continued

F / Accuracy and scope

F1. What should be handed to the dentist?

D. The mismatch between the expected filling and recorded consultation, plus the patient's questions

The dentist needs the actual expectation and questions to clarify the visit without fabricated consent or diagnosis.

Return to the exercise, page 14

F2. What does attendance establish by itself?

C. The patient came to the appointment, not agreement to every procedure

Attendance is an event and does not replace the required explanation and consent process.

Return to the exercise, page 14

F / Guided speaking

1. review

Assistant: The recorded appointment type is a review.

Turn 1. The schedule states a review rather than a confirmed fitting procedure.

2. crown fitting

Patient: I expected a crown fitting.

Turn 2. Crown fitting is the patient's stated expectation, not the confirmed booking.

3. twenty

Assistant: The allocated time is twenty minutes.

Turn 3. The supplied schedule allocates twenty minutes and does not guarantee a procedure.

4. unconfirmed

Assistant: Fitting readiness remains unconfirmed.

Turn 4. The dentist has not confirmed readiness, so the assistant must preserve that status.

Return to the conversation, page 15

01 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "A consultation is not a confirmed filling". Lena: "If I stay to talk to the dentist, am I agreeing to go ahead with whatever they recommend during that conversation?" Amir: [missing response] Lena: "Thank you. I want the dentist to know I expected treatment, but I do not want anyone to think I am demanding a procedure that might not be right." Which response belongs in the gap?

A. No. Informed consent concerns the actual care being proposed and discussed. Attending this visit does not mean you have already accepted every possible procedure or cost.

A Correct. This is Amir's response in turn 14. It connects Lena's preceding point with Lena's follow-up. Informed consent requires the applicable explanation and agreement process; staying for a consultation is not blanket treatment permission.

B Not here. This line belongs to turn 16, after Lena says: "Thank you. I want the dentist to know I expected treatment, but I do not want anyone to think I am demanding a procedure that might not be right." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 20, after Lena says: "All right, I will speak to the dentist first. Please mention the time off work too. Another appointment would take some arranging." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 10, after Lena says: "I would also like to understand the cost before agreeing. I received a number over the phone, but I do not know exactly what it covered." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 15

02 / Answers and explanations

A / Read the situation

A1. What is known about P18?

C. It is present, but required readiness documentation cannot be confirmed.

The brief establishes an unresolved verification gap, not a confirmed failure or completed readiness.

Return to the exercise, page 16

A2. What may Bruno provide?

B. A replacement whose readiness has been verified

A verified replacement may be available, but availability itself has not yet been confirmed.

Return to the exercise, page 16

A3. What must not happen?

D. Treating pack presence as permission to skip the required checks

The assistant lacks authority to waive checks, and physical presence does not establish readiness.

Return to the exercise, page 16

D / Response choices

D1. Which message is best?

D. Please pause; P18's required readiness record is unconfirmed, and I need the approved review or a verified replacement.

The message identifies the item, gap, and appropriate response without guessing a result or waiving the process.

Return to the exercise, page 20

D2. What does a color change alone establish here?

B. Not completion of every required readiness check

A single observation does not replace the complete applicable verification process.

Return to the exercise, page 20

02 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Removal of soil and material, distinct from sterilization. Which term fits?

A. cleaning

A Correct. cleaning: Removal of soil and material, distinct from sterilization. The usual combination is "verify the cleaning step".

B Not here. load number: The identifier linking items to a particular processing load. The usual combination is "verify the load number".

C Not here. IFU: Instructions for use; the relevant manufacturer's directions. The usual combination is "consult the IFU".

D Not here. hold: A status preventing use pending the appropriate resolution. The usual combination is "maintain the hold".

Return to the exercise, page 20

D4. Rebuild the exchange from "The pack is present; readiness is not verified". Tessa: "The package is here, but I do not want to say it is contaminated. The issue I can establish is that the required documentation cannot be confirmed." Bruno: [missing response] Tessa: "Would a replacement solve the immediate setup problem? I heard there might be another pack, but I have not checked its identity or status." Which response belongs in the gap?

D. That is the right distinction. We need the readiness documentation, not an invented finding of failure or a reassuring assumption. The uncertainty still means we must not bypass the check.

A Not here. This line belongs to turn 20, after Tessa says: "Understood. I will record the actual pack status, whom I contacted, and any verified replacement details, without claiming a failure that has not been found." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 4, after Tessa says: "It has not been used. I have not released it, and I do not have authority to waive the missing check just because the room is busy." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 14, after Tessa says: "Someone asked whether the visible indicator was enough. I was unsure how to explain why one visible feature did not settle every readiness question." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Bruno's response in turn 6. It connects Tessa's preceding point with Tessa's follow-up. Readiness documentation supports the required verification; its absence is a gap, not by itself a confirmed contamination finding.

Return to the exercise, page 20

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02 / Answers and explanations / continued

E / Conversation

1. instrument pack

Turn 2. The instrument pack identifier makes the concern specific and prevents confusion with another item or replacement.

2. release status

Turn 4. Release status remains unconfirmed because the required checks have not been established; a busy schedule does not authorize use.

3. readiness documentation

Turn 6. Readiness documentation supports the required verification; its absence is a gap, not by itself a confirmed contamination finding.

4. verified replacement

Turn 8. A verified replacement must have its own readiness established; merely finding another package is insufficient.

5. Traceability

Turn 10. Traceability connects the particular pack to its relevant processing history rather than accepting an unrelated record.

6. load number

Turn 12. The load number identifies the relevant processing load; substituting another number would break the factual link.

7. chemical indicator

Turn 14. A chemical indicator relates to specified process exposure and does not independently establish every readiness requirement.

8. hold

Turn 16. The hold prevents premature use while the identified verification issue is being resolved through the required process.

9. cycle record

Turn 18. The cycle record concerns the original processing history, which still needs the appropriate review even if a replacement is available.

02 / Answers and explanations / continued

10. IFU

Turn 20. The instructions for use are the manufacturer's applicable directions and remain part of the required process, not an optional reference.

Return to the conversation, page 22

F / Accuracy and scope

F1. What should happen to the original issue if a replacement is found?

D. It remains subject to the required review and record process.

A replacement supports the immediate workflow but does not establish or erase the original pack's history.

Return to the exercise, page 24

F2. Which statement is precise?

C. Readiness is unverified; no confirmed failure is established in the supplied facts.

The statement preserves uncertainty while avoiding both a fabricated failure and an unsupported release.

Return to the exercise, page 24

F / Guided speaking

1. Q7

Assistant: The affected pack is Q7.

Turn 1. Q7 is the specific item whose readiness remains unresolved.

2. not used

Lead: Its use status is not used.

Turn 2. The brief explicitly states that the pack has not been used.

3. Eva

Assistant: The review contact is Eva.

Turn 3. Eva is the lead assistant identified for the review.

4. verified

Lead: The replacement is requested but not yet verified.

Turn 4. A replacement request does not establish the alternative item's readiness.

Return to the conversation, page 25

02 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "The pack is present; readiness is not verified". Tessa: "Someone asked whether the visible indicator was enough. I was unsure how to explain why one visible feature did not settle every readiness question." Bruno: [missing response] Tessa: "I will tell the dentist that I have raised the specific gap and that you are checking the record and a possible verified alternative." Which response belongs in the gap?

A. A chemical indicator has a defined purpose, but it is not a substitute for all applicable checks. We need the practice process and manufacturer instructions followed, not an isolated shortcut.

A Correct. This is Bruno's response in turn 14. It connects Tessa's preceding point with Tessa's follow-up. A chemical indicator relates to specified process exposure and does not independently establish every readiness requirement.

B Not here. This line belongs to turn 6, after Tessa says: "The package is here, but I do not want to say it is contaminated. The issue I can establish is that the required documentation cannot be confirmed." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 4, after Tessa says: "It has not been used. I have not released it, and I do not have authority to waive the missing check just because the room is busy." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 20, after Tessa says: "Understood. I will record the actual pack status, whom I contacted, and any verified replacement details, without claiming a failure that has not been found." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 25

03 / Answers and explanations

A / Read the situation

A1. Which word did Noor hear?

B. Tray

The brief says tray was audible while the distinguishing part was missed.

Return to the exercise, page 28

A2. What are the two listed possibilities?

A. Instrument tray and impression tray

Both tray types appear on the setup list, making the incomplete request ambiguous.

Return to the exercise, page 28

A3. What does the dentist clarify?

D. The instrument tray is wanted for the current setup.

The clarification resolves the current item while leaving ordinary practice procedures in place.

Return to the exercise, page 28

D / Response choices

D1. Which question best resolves the actual ambiguity?

B. Did you mean the instrument tray or the impression tray?

The question identifies the two relevant alternatives and requests the missing distinction.

Return to the exercise, page 32

D2. Which read-back is complete for the clarified request?

A. The instrument tray for the current setup, correct?

The complete read-back preserves the distinguishing item and current context.

Return to the exercise, page 32

03 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: A device used to hold impression material under the appropriate procedure. Which term fits?

A. impression tray

A Correct. impression tray: A device used to hold impression material under the appropriate procedure. The usual combination is "identify the impression tray".

B Not here. bur: A rotary cutting or finishing attachment with a particular design and purpose. The usual combination is "verify the bur identifier".

C Not here. periodontal probe: An instrument for periodontal measurements under the relevant clinical procedure. The usual combination is "distinguish the periodontal probe".

D Not here. read-back: Repeating an instruction to check it was heard correctly. The usual combination is "give a complete read-back".

Return to the exercise, page 32

D4. Rebuild the exchange from "Which tray did you mean?". Noor: "I heard suction from the doorway earlier. Do you mean the HVE rather than the saliva ejector? Both are on the equipment list." Dr. Shaw: [missing response] Noor: "HVE confirmed. Marta has asked which tray to bring. I will give her the full item name and tell her this is for the current setup." Which response belongs in the gap?

D. Yes, HVE. That is the distinguishing detail I left out earlier. Please keep those names separate when you pass the request to Marta.

A Not here. This line belongs to turn 16, after Noor says: "Marta is here now. She has repeated instrument tray and HVE. She also wants to confirm that you did not request impression equipment at this stage." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 18, after Noor says: "I have repeated that to Marta, and she has confirmed it back. She is checking the tray's readiness and will report before bringing it across." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 4, after Noor says: "Instrument tray for the current setup, correct? I will check its readiness status before bringing it over. The other tray will stay where it is." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Dr. Shaw's response in turn 6. It connects Noor's preceding point with Noor's follow-up. Naming HVE distinguishes the requested high-volume evacuator from the listed saliva ejector.

Return to the exercise, page 32

E / Conversation

1. interruption

Turn 2. The interruption masked part of the request; it does not justify choosing an item by guesswork.

2. read-back

Turn 4. The read-back repeats the item and timing so the dentist can confirm the intended request.

03 / Answers and explanations / continued

3. distinguishing detail

Turn 6. Naming HVE distinguishes the requested high-volume evacuator from the listed saliva ejector.

4. instrument tray

Turn 8. Instrument tray is the requested item, rather than the separate impression tray on the list.

5. ambiguous instruction

Turn 10. Small suction does not reliably identify the device or whether this handwritten request is current.

6. setup briefing

Turn 12. The setup briefing is the appropriate point to reconcile the equipment list before relying on it.

7. chairside

Turn 14. Chairside exchanges take place beside the dental chair, where short requests still need precise identifying details.

8. impression tray

Turn 16. The impression tray is specifically excluded from this current request, not declared unnecessary for every appointment.

9. closed-loop request

Turn 18. The receiver repeats the specific request and the requester confirms it, closing the communication loop.

10. handoff

Turn 20. The handoff transfers the confirmed items and the still-unresolved note without concealing either status.

Return to the conversation, page 34

F / Accuracy and scope

F1. What should Noor do with unfamiliar local shorthand?

D. Ask for the full name and confirm the intended meaning.

Local shorthand can vary, so explicit clarification is more reliable than assumed meaning.

Return to the exercise, page 36

03 / Answers and explanations / continued

F2. What does confirmation of the request establish?

C. Shared understanding of the requested item, not completion of all readiness checks

A communication check resolves the instruction while separate procedural requirements remain applicable.

Return to the exercise, page 36

F / Guided speaking

1. device name

Assistant: I heard suction but missed the device name.

Turn 1. The missing information is the distinguishing device name, not the general word suction.

2. HVE

Dentist: The requested device is HVE.

Turn 2. The dentist specifically clarifies HVE rather than the saliva ejector.

3. read-back

Assistant: I will confirm it with a complete read-back.

Turn 3. Repeating the complete request lets the dentist confirm or correct the understanding.

4. required

Dentist: The ordinary readiness checks remain required.

Turn 4. Clarifying the device does not waive separate readiness requirements.

Return to the conversation, page 37

03 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Which tray did you mean?". Noor: "For any size-specific request, please include the size in the full name. I missed a modifier in yesterday's briefing and had to ask again." Dr. Shaw: [missing response] Noor: "Marta is here now. She has repeated instrument tray and HVE. She also wants to confirm that you did not request impression equipment at this stage." Which response belongs in the gap?

A. Of course. In a chairside exchange, say which part you missed. I would rather repeat one size than discover that we understood different items.

A Correct. This is Dr. Shaw's response in turn 14. It connects Noor's preceding point with Noor's follow-up. Chairside exchanges take place beside the dental chair, where short requests still need precise identifying details.

B Not here. This line belongs to turn 4, after Noor says: "Instrument tray for the current setup, correct? I will check its readiness status before bringing it over. The other tray will stay where it is." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 20, after Noor says: "My note reads: instrument tray and HVE for this setup, checks required; small suction note awaiting clarification. Marta has those same details." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 6, after Noor says: "I heard suction from the doorway earlier. Do you mean the HVE rather than the saliva ejector? Both are on the equipment list." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 37

04 / Answers and explanations

A / Read the situation

A1. What does Marco ask?

A. Whether the image proves that a tooth needs treatment

The question concerns interpretation and treatment significance of the image.

Return to the exercise, page 38

A2. What is Suki not authorized to do here?

B. Interpret the image or recommend treatment

The fictional role expressly excludes image interpretation and treatment recommendation.

Return to the exercise, page 38

A3. What clinical finding is established in the brief?

B. None

The brief supplies a patient question but no interpreted finding or diagnosis.

Return to the exercise, page 38

D / Response choices

D1. Which response is appropriate?

C. I cannot interpret it, but I will ask the dentist to explain whether it supports treatment and why.

The response preserves the treatment question and routes interpretation to the appropriate clinician.

Return to the exercise, page 42

D2. What is wrong with labeling an unexplained feature a cavity?

C. No interpreted finding is supplied, and Suki lacks authorization to diagnose it.

The proposed label invents a clinical conclusion that neither the facts nor the assistant's role supports.

Return to the exercise, page 42

04 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: In dental radiography, an image obtained with the receptor outside the mouth. Which term fits?

A. extraoral image

A Correct. extraoral image: In dental radiography, an image obtained with the receptor outside the mouth. The usual combination is "identify an extraoral image".

B Not here. radiopaque: Appearing relatively lighter on a radiograph because fewer X-rays reach the receptor. The usual combination is "recognize the term radiopaque".

C Not here. image receptor: The film or digital sensor receiving the image information. The usual combination is "identify the image receptor".

D Not here. comparison image: An earlier or other image used for a defined comparison. The usual combination is "identify the comparison image".

Return to the exercise, page 42

D4. Rebuild the exchange from "Explain the route to an answer, not the image". Marco: "Is darker always worse? I have heard words used for dark and light areas, but I do not know whether they describe a disease." Suki: [missing response] Marco: "Then I should ask what the feature actually represents. I would also like to know whether anything else in the examination supports the same conclusion." Which response belongs in the gap?

A. Terms such as radiolucent describe image appearance, not a diagnosis by themselves. The dentist can explain the relevant terminology in context rather than have us attach a condition to an isolated word.

A Correct. This is Suki's response in turn 6. It connects Marco's preceding point with Marco's follow-up. Radiolucent describes relative image appearance and is not, by itself, the name of a disease or treatment decision.

B Not here. This line belongs to turn 10, after Marco says: "There is also a mark on the screen. I cannot tell whether it is part of the original image or something someone added to point out an area." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 8, after Marco says: "Then I should ask what the feature actually represents. I would also like to know whether anything else in the examination supports the same conclusion." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 2, after Marco says: "Suki, I can see a darker area on this picture. Does that prove the tooth needs treatment, or am I misunderstanding what I am looking at?" It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 42

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04 / Answers and explanations / continued

E / Conversation

1. diagnostic significance

Turn 2. Diagnostic significance concerns what the image means for identifying a condition, which requires the appropriate clinical explanation.

2. radiograph

Turn 4. A radiograph is the X-ray image; displaying it does not establish authority to diagnose or explain every feature.

3. radiolucent

Turn 6. Radiolucent describes relative image appearance and is not, by itself, the name of a disease or treatment decision.

4. correlation

Turn 8. Correlation here means considering the image alongside other relevant clinical information, not treating one feature as sufficient proof.

5. image annotation

Turn 10. An image annotation is an added label or mark whose purpose and source need explanation rather than assumed diagnostic meaning.

6. artifact

Turn 12. An artifact is a feature introduced by a technical or other non-anatomical factor; its presence here has not been established.

7. authorized interpreter

Turn 14. The authorized interpreter is the appropriately qualified professional who can assess the image within the actual clinical and legal framework.

8. treatment indication

Turn 16. The treatment indication is the clinical reason for considering treatment, which must be explained rather than inferred from merely having an image.

9. comparison image

Turn 18. A comparison image is another image used for a defined clinical comparison; the scenario does not establish that one was reviewed.

04 / Answers and explanations / continued

10. image interpretation

Turn 20. Image interpretation is the professional assessment the patient is requesting; the assistant's handoff supports but does not supply it.

Return to the conversation, page 44

F / Accuracy and scope

F1. Which distinction is necessary?

A. Producing an image and interpreting it are different activities.

Tasks connected with imaging have different competencies and permissions, so they cannot be treated as interchangeable.

Return to the exercise, page 47

F2. Which question should reach the dentist?

D. Does the image support treatment, and how does it relate to the examination and options?

The question seeks the actual clinical significance and options rather than an unsupported shortcut.

Return to the exercise, page 47

F / Guided speaking

1. radiopaque

Assistant: The term in your question is radiopaque.

Turn 1. Radiopaque is the supplied image-appearance term, not a confirmed treatment indication.

2. crown

Patient: I want to know whether I need a crown.

Turn 2. The patient asks about a crown, but the question does not establish that it is needed.

3. dentist

Assistant: The explanation needs the dentist.

Turn 3. The dentist is the identified clinician who can explain the note and its significance.

4. established

Assistant: No treatment conclusion has been established here.

Turn 4. The brief supplies no interpreted finding or confirmed treatment decision.

Return to the conversation, page 48

04 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Explain the route to an answer, not the image". Marco: "I appreciate that. I do not want reassurance if it is not justified, but I also do not want a frightening answer based on a guess." Suki: [missing response] Marco: "If a treatment is recommended, I want to understand why that treatment follows from the evidence rather than think the picture makes the decision automatically." Which response belongs in the gap?

C. The authorized interpreter can explain what is established and what remains uncertain. I can make sure your concern is heard without telling you that everything is normal or definitely abnormal.

A Not here. This line belongs to turn 10, after Marco says: "There is also a mark on the screen. I cannot tell whether it is part of the original image or something someone added to point out an area." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 8, after Marco says: "Then I should ask what the feature actually represents. I would also like to know whether anything else in the examination supports the same conclusion." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Suki's response in turn 14. It connects Marco's preceding point with Marco's follow-up. The authorized interpreter is the appropriately qualified professional who can assess the image within the actual clinical and legal framework.

D Not here. This line belongs to turn 20, after Marco says: "Please ask what the patch means and whether it matches the examination. That is what I need to understand first." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 48

05 / Answers and explanations

A / Read the situation

A1. What does Ava ask Ben to promise?

D. That the procedure will not hurt

The stated request is a guarantee about pain, not a confirmed comfort plan.

Return to the exercise, page 51

A2. What has not happened yet?

A. The dentist's discussion of comfort measures and Ava's concerns

The brief says the clinical comfort discussion has not yet taken place.

Return to the exercise, page 51

A3. What can Ben appropriately arrange?

B. Communication of the concern and a dentist explanation of the plan and pause signal

Ben can support the clinical conversation without inventing medication, sensations, or a treatment plan.

Return to the exercise, page 51

D / Response choices

D1. Which reassurance is appropriate?

D. I will help you discuss pain concerns and the comfort plan with the dentist before starting.

The response offers concrete support without a false sensation guarantee or unauthorized clinical decision.

Return to the exercise, page 55

D2. Which statement wrongly treats consent as permanent?

A. You agreed once, so you cannot ask to stop.

An earlier agreement does not remove the need to address ongoing consent and a stop request.

Return to the exercise, page 55

05 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: A question or fear about pain that needs to be heard and addressed. Which term fits?

B. pain concern

A Not here. reassurance: Information or support intended to reduce concern without misleading. The usual combination is "give honest reassurance".

B Correct. pain concern: A question or fear about pain that needs to be heard and addressed. The usual combination is "relay a pain concern".

C Not here. topical anesthetic: An anesthetic applied to a surface for an appropriate clinical purpose. The usual combination is "clarify a topical anesthetic question".

D Not here. trigger: Something associated with an anxiety or distress response. The usual combination is "ask about a reported trigger".

Return to the exercise, page 55

D4. Rebuild the exchange from "Reassure without promising a painless procedure". Ava: "Last time I did not know whether I was supposed to feel pressure or pain. I could not tell what the person expected me to report." Ben: [missing response] Ava: "I would like to know what can be done to help me feel comfortable. I do not know which choices are appropriate for me." Which response belongs in the gap?

C. We can ask about the expected sensation in this procedure. I should not tell you which feeling is acceptable without the clinician's explanation or use the word pressure to dismiss what you report.

A Not here. This line belongs to turn 2, after Ava says: "Ben, I barely slept. Last time I felt something sharp and could not get anyone's attention. Can you promise this will not hurt?" It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 16, after Ava says: "If I agree to continue after the explanation and then change my mind, I want to know that I can still say something." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Ben's response in turn 6. It connects Ava's preceding point with Ava's follow-up. Sensation refers to what the patient may feel; the actual clinician must explain it in relation to the planned procedure.

D Not here. This line belongs to turn 12, after Ava says: "Please tell them about my previous experience too. I was not sure how to ask for a break once the person had started working." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 55

E / Conversation

1. dental anxiety

Turn 2. Dental anxiety identifies the patient's apprehension and should be acknowledged rather than dismissed or treated as a diagnosis of the tooth.

05 / Answers and explanations / continued

2. reassurance

Turn 4. Reassurance should provide honest support and information, not pressure the patient to suppress a concern.

3. sensation

Turn 6. Sensation refers to what the patient may feel; the actual clinician must explain it in relation to the planned procedure.

4. comfort measure

Turn 8. A comfort measure must be appropriate to the patient's circumstances and explained by the responsible clinical team.

5. Sedation

Turn 10. Sedation and local anesthesia are different clinical concepts; neither is confirmed or selected in this scenario.

6. pause signal

Turn 12. A pause signal is an agreed communication method whose meaning and safe response must be confirmed for the actual care.

7. communication preference

Turn 14. The communication preference is the patient's request for explanation before each stage, not a demand for a particular clinical treatment.

8. ongoing consent

Turn 16. Ongoing consent requires attention throughout care; an earlier yes does not erase later questions or a request to stop.

9. absolute guarantee

Turn 18. An absolute guarantee about pain would claim certainty the assistant does not have and could undermine honest communication.

10. clinician review

Turn 20. Clinician review supplies the qualified discussion of the actual plan; the assistant's acknowledgment alone does not complete it.

Return to the conversation, page 57

05 / Answers and explanations / continued

F / Accuracy and scope

F1. What should be confirmed about a pause signal?

B. The actual signal and how the team will respond safely

A useful signal requires shared understanding of both the request and the team's response.

Return to the exercise, page 60

F2. What should the record distinguish?

B. Reported concern, explanation given, and any agreement actually reached

Accurate recording separates concerns, communication, and actual decisions rather than assuming an outcome.

Return to the exercise, page 60

F / Guided speaking

1. each stage

Assistant: The requested communication preference is explanation before each stage.

Turn 1. The patient specifically asks for explanations before each stage of care.

2. pause signal

Patient: I would like to agree a pause signal.

Turn 2. The request concerns how to communicate a need for a pause.

3. confirmed

Assistant: The signal is not yet confirmed.

Turn 3. The clinician has not yet agreed the signal or its response.

4. guarantee

Assistant: I cannot provide an absolute guarantee about sensations.

Turn 4. The supplied role does not support an unqualified promise about what the patient will feel.

Return to the conversation, page 61

05 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Reassure without promising a painless procedure". Ava: "I would also like an explanation before each stage. Surprises make me tense, even when the action itself might not be difficult." Ben: [missing response] Ava: "If I agree to continue after the explanation and then change my mind, I want to know that I can still say something." Which response belongs in the gap?

D. I can pass on that communication preference. The team can discuss how to explain the sequence clearly and check what information helps you feel prepared.

A Not here. This line belongs to turn 4, after Ava says: "People say not to worry, but that makes me feel I should stay quiet if something bothers me." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 10, after Ava says: "Someone mentioned sedation to me, although I do not know whether they meant the same thing as having part of the mouth numb." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 8, after Ava says: "I would like to know what can be done to help me feel comfortable. I do not know which choices are appropriate for me." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Ben's response in turn 14. It connects Ava's preceding point with Ava's follow-up. The communication preference is the patient's request for explanation before each stage, not a demand for a particular clinical treatment.

Return to the exercise, page 61

06 / Answers and explanations

A / Read the situation

A1. Which mismatch is supplied?

A. The list ends 462 and the folder label ends 426.

The two supplied identifier endings differ despite the shared surname.

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A2. Which record is established as correct?

D. Neither

The brief does not establish the correct source, so the approved process must resolve the mismatch.

Return to the exercise, page 62

A3. What has Diego not done?

B. Used the folder or disclosed its contents

The brief says the folder has not been used and its contents have not been disclosed.

Return to the exercise, page 62

D / Response choices

D1. Which report is precise?

B. The list ends 462, the folder ends 426, and I have not used the folder.

The precise report preserves the values and action status without assigning unsupported correctness.

Return to the exercise, page 66

D2. Why not merge the records immediately?

C. A shared surname does not establish that they concern the same person.

Different people can share a surname, so consolidation without verification could combine their information incorrectly.

Return to the exercise, page 66

06 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: A disagreement between relevant identifying information. Which term fits?

B. identifier mismatch

A Not here. dentition: The arrangement or set of teeth present. The usual combination is "distinguish dentition references".

B Correct. identifier mismatch: A disagreement between relevant identifying information. The usual combination is "report an identifier mismatch".

C Not here. folder label: The identifying information shown on a physical folder. The usual combination is "check the folder label".

D Not here. access control: Restriction of information access to authorized users and purposes. The usual combination is "follow access controls".

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D4. Rebuild the exchange from "A shared surname is not a verified match". Diego: "Four-six-two and four-two-six look like swapped digits. I nearly trusted the screen over the folder, but I have no evidence that the screen is right." Priya: [missing response] Diego: "I will preserve the two values and the fact that the folder has not been used. That seems more helpful than deciding the cause in advance." Which response belongs in the gap?

B. An identifier mismatch does not tell us which source is right. A plausible explanation still needs verification; the screen does not become authoritative merely because it is easier to read.

A Not here. This line belongs to turn 2, after Diego says: "Priya, I have an identifier mismatch before using this folder. The appointment list ends four-six-two, but the folder label ends four-two-six." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Priya's response in turn 6. It connects Diego's preceding point with Diego's follow-up. An identifier mismatch establishes disagreement between sources, not which source is correct or why the difference occurred.

C Not here. This line belongs to turn 20, after Diego says: "Until the check is complete, my update will say that the match is unverified, the folder has not been used, and you are coordinating the review." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 12, after Diego says: "The folder also contains a dental chart, but I have not used its contents to guess whether it looks like the patient I remember." It addresses that point, not the missing turn in the supplied exchange.

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06 / Answers and explanations / continued

E / Conversation

1. source record

Turn 2. The source record preserves the original information needed to investigate the mismatch rather than replacing it with a guess.

2. name collision

Turn 4. A name collision means more than one person shares a name; it does not prove the records should be combined.

3. identifier mismatch

Turn 6. An identifier mismatch establishes disagreement between sources, not which source is correct or why the difference occurred.

4. wrong-patient risk

Turn 8. Wrong-patient risk arises when an unverified record could be used for the wrong person; raising the mismatch before use supports prevention.

5. patient identifier

Turn 10. Patient identifiers must be verified through the actual approved process, not by treating a prompted guess as reliable confirmation.

6. odontogram

Turn 12. An odontogram records dental information and must not become an informal substitute for the required patient-identification checks.

7. traceable correction

Turn 14. A traceable correction preserves the verified basis and history of a change rather than silently overwriting the discrepancy.

8. record merge

Turn 16. A record merge combines information and therefore requires verified identity and the appropriate authority, not a shared surname alone.

9. confidential handoff

Turn 18. A confidential handoff shares the relevant information through an appropriately protected route instead of exposing it to unrelated people.

06 / Answers and explanations / continued

10. audit trail

Turn 20. The audit trail records relevant actions and changes so the actual review and correction history can be understood later.

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F / Accuracy and scope

F1. What should a legitimate correction preserve?

A. The verified basis and appropriate traceable history

A traceable correction records the supported change rather than concealing how the discrepancy was resolved.

Return to the exercise, page 71

F2. What are the three-digit endings in this exercise?

D. Teaching references, not a complete real-world identification method

The exercise uses shortened references for language practice and does not replace an approved full identification process.

Return to the exercise, page 71

F / Guided speaking

1. 815

Assistant: The schedule reference ends 815.

Turn 1. The supplied schedule value is 815, distinct from the folder value.

2. 851

Lead: The folder reference ends 851.

Turn 2. The supplied folder value is 851 and must not be silently changed.

3. not been used

Assistant: The folder has not been used.

Turn 3. The brief places the discrepancy before any use of the folder.

4. unverified

Lead: The match remains unverified.

Turn 4. Neither source is established as correct, so the identification process remains necessary.

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06 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "A shared surname is not a verified match". Diego: "If the review shows that one label is wrong, I should not remove the old information without recording how and why it was changed." Priya: [missing response] Diego: "And if the review reveals two records for one person, that would still be a separate authorized process rather than something I do because the surname matches." Which response belongs in the gap?

B. A traceable correction is needed under the record procedure. The result must show the supported amendment, not pretend that the original discrepancy never existed.

A Not here. This line belongs to turn 16, after Diego says: "And if the review reveals two records for one person, that would still be a separate authorized process rather than something I do because the surname matches." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Priya's response in turn 14. It connects Diego's preceding point with Diego's follow-up. A traceable correction preserves the verified basis and history of a change rather than silently overwriting the discrepancy.

C Not here. This line belongs to turn 4, after Diego says: "There are two patients named Morgan. I have not used the folder or disclosed its contents, and I cannot establish which identifier belongs with this visit." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 18, after Diego says: "Can we check the details in the staff area? Both patients' names are visible here, and I do not want this conversation carrying into the waiting room." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 72

07 / Answers and explanations

A / Read the situation

A1. What is the laboratory estimate?

B. Tuesday by noon, not guaranteed

The lab estimates arrival by Tuesday noon and explicitly does not guarantee it.

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A2. When can the lab provide the next status update?

B. Monday at 16:00

Monday 16:00 is the update time, not the estimated delivery time.

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A3. What else is required before proceeding?

B. Dentist confirmation of readiness

The scenario separately requires the dentist to confirm readiness after the relevant checks.

Return to the exercise, page 75

D / Response choices

D1. Which patient update is accurate?

B. The lab estimates Tuesday noon delivery; the practice still needs receipt and dentist readiness confirmation.

The accurate message separates estimated arrival from the practice's required readiness decision.

Return to the exercise, page 79

D2. What is the interval from noon to 14:00?

C. Two hours, with no proof that this is sufficient review time

The arithmetic gives two hours, but the brief supplies no rule making that sufficient.

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07 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Approval for the specified laboratory task. Which term fits?

A. work authorization

A Correct. work authorization: Approval for the specified laboratory task. The usual combination is "confirm work authorization".

B Not here. crown: A restoration covering the relevant tooth structure as clinically prescribed. The usual combination is "track a crown case".

C Not here. receipt confirmation: Verification that the specified item has arrived at the destination. The usual combination is "obtain receipt confirmation".

D Not here. occlusal check: Clinical assessment relating to how opposing teeth or restorations meet. The usual combination is "refer the occlusal check".

Return to the exercise, page 79

D4. Rebuild the exchange from "Expected by noon is not ready for two". Rina: "What can you tell us at Monday's four-o'clock update? It would help to separate what is finished from what has actually left the laboratory." Felix: [missing response] Rina: "If it has been sent, we also need the correct reference and tracking details. We sometimes receive more than one case in the same delivery." Which response belongs in the gap?

C. We can report the production status and whether dispatch has occurred, along with the current delivery estimate. I will not call an item shipped simply because production is expected to finish.

A Not here. This line belongs to turn 8, after Rina says: "If it has been sent, we also need the correct reference and tracking details. We sometimes receive more than one case in the same delivery." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 18, after Rina says: "Let me read those back: update Monday at four; estimated arrival Tuesday by noon. I will put them in separate fields so nobody confuses the dates." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Felix's response in turn 6. It connects Rina's preceding point with Rina's follow-up. Production status identifies the manufacturing stage and must remain distinct from actual dispatch or delivery.

D Not here. This line belongs to turn 14, after Rina says: "If something changes in the clinical specification, I cannot simply amend it over this call to make the production schedule easier." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 79

07 / Answers and explanations / continued

E / Conversation

1. estimated delivery

Turn 2. Estimated delivery is a forecast of Tuesday noon arrival, not a guaranteed appointment or confirmed receipt.

2. clinical readiness

Turn 4. Clinical readiness requires the dentist's relevant checks and decision; a forecast delivery time does not establish it.

3. production status

Turn 6. Production status identifies the manufacturing stage and must remain distinct from actual dispatch or delivery.

4. case reference

Turn 8. The case reference links the status and tracking to C62 rather than another case in the same delivery.

5. tracking event

Turn 10. A tracking event records a carrier action and is not necessarily confirmation that the intended case was received by the practice.

6. Receipt confirmation

Turn 12. Receipt confirmation verifies arrival of the specified item; it remains separate from the clinical assessment needed before proceeding.

7. laboratory prescription

Turn 14. The laboratory prescription contains authorized clinical instructions and cannot be altered casually to accommodate a schedule.

8. contingency booking

Turn 16. A contingency booking is an alternative appointment arrangement for the practice to consider, not an automatic cancellation or confirmed replacement.

9. status checkpoint

Turn 18. The status checkpoint is the agreed Monday update time and must not be confused with the Tuesday delivery estimate.

07 / Answers and explanations / continued

10. dispatch status

Turn 20. Dispatch status records whether the case has actually been sent; it is one logistical fact, not final appointment approval.

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F / Accuracy and scope

F1. Which request is most useful at Monday's checkpoint?

B. Current production, actual dispatch, revised estimate, and any change affecting delivery

The requested status gives the practice actionable facts without replacing clinical authority or current evidence.

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F2. What does a delivered status not establish by itself?

B. Clinical readiness to proceed

A delivery event concerns logistics and does not supply the dentist's separate readiness confirmation.

Return to the exercise, page 84

F / Guided speaking

1. D14

Assistant: The case reference is D14.

Turn 1. D14 identifies the particular laboratory case in this exchange.

2. Wednesday at 10:00

Lab: Estimated arrival is Wednesday at 10:00.

Turn 2. The supplied estimate concerns Wednesday arrival and is explicitly not guaranteed.

3. Tuesday at 15:00

Assistant: The status update is due Tuesday at 15:00.

Turn 3. Tuesday 15:00 is the information checkpoint, not the delivery time.

4. clinical readiness

Lab: The dentist must confirm clinical readiness.

Turn 4. Delivery timing alone does not establish readiness for the appointment.

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07 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Expected by noon is not ready for two". Rina: "If something changes in the clinical specification, I cannot simply amend it over this call to make the production schedule easier." Felix: [missing response] Rina: "The patient may need time to plan travel. I will ask the practice how to communicate the uncertainty and any alternative appointment arrangements." Which response belongs in the gap?

D. Any change to the laboratory prescription needs the appropriate authorization and documentation. We can communicate a question, but the convenience of a delivery date does not decide clinical specifications.

A Not here. This line belongs to turn 18, after Rina says: "Let me read those back: update Monday at four; estimated arrival Tuesday by noon. I will put them in separate fields so nobody confuses the dates." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 8, after Rina says: "If it has been sent, we also need the correct reference and tracking details. We sometimes receive more than one case in the same delivery." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 6, after Rina says: "What can you tell us at Monday's four-o'clock update? It would help to separate what is finished from what has actually left the laboratory." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Felix's response in turn 14. It connects Rina's preceding point with Rina's follow-up. The laboratory prescription contains authorized clinical instructions and cannot be altered casually to accommodate a schedule.

Return to the exercise, page 85

08 / Answers and explanations

A / Read the situation

A1. What does Owen need?

A. Clarification of a phrase in his actual postoperative instructions

The caller asks what a particular written phrase means in his circumstances.

Return to the exercise, page 86

A2. What is outside Farah's supplied role?

B. Reinterpreting or changing the instructions

The brief explicitly excludes independent reinterpretation and alteration of clinical instructions.

Return to the exercise, page 86

A3. What overrides an ordinary callback queue?

D. The appropriate route for an urgent or emergency concern

Urgent concerns require their designated response rather than delay for routine clarification.

Return to the exercise, page 86

D / Response choices

D1. Which response is appropriate?

C. Please quote the phrase; I will route the exact question for clinical clarification without changing it.

The response preserves the question and routes it within the assistant's role.

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D2. What should Farah say about callback timing?

D. No time is confirmed yet.

The brief supplies no confirmed callback time, so neither a specific promise nor an indefinite safety claim is justified.

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08 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: An instruction recorded in text whose actual wording matters. Which term fits?

B. written direction

A Not here. infection concern: A reported worry about possible infection, not itself a diagnosis. The usual combination is "route an infection concern".

B Correct. written direction: An instruction recorded in text whose actual wording matters. The usual combination is "quote the written direction".

C Not here. review interval: The specified period before reassessment or follow-up. The usual combination is "confirm the review interval".

D Not here. contact verification: Checking relevant contact details through the approved process. The usual combination is "complete contact verification".

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D4. Rebuild the exchange from "An unclear instruction needs an exact clinical answer". Owen: "I am glad you said that. I do not need a general answer copied from someone else's visit; I need to know what the direction means for mine." Farah: [missing response] Owen: "Will the dentist actually see that wording? Last time I was told a message had been logged, but I still did not know what to do." Which response belongs in the gap?

D. That requires clinical clarification. I can pass the exact wording and your question through the practice callback process, using the relevant verified record and contact details.

A Not here. This line belongs to turn 2, after Owen says: "Farah, I am home with the instruction sheet, but one sentence has me stuck. Could someone explain it before I guess what I am supposed to do?" It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 8, after Owen says: "Will the dentist actually see that wording? Last time I was told a message had been logged, but I still did not know what to do." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 18, after Owen says: "And if the clinician changes the direction, I would like the new wording clear. I do not want two different instructions without knowing which applies." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Farah's response in turn 6. It connects Owen's preceding point with Owen's follow-up. Clinical clarification provides the appropriate patient-specific explanation of the instruction rather than an assistant's improvised interpretation.

Return to the exercise, page 91

08 / Answers and explanations / continued

E / Conversation

1. written direction

Turn 2. The written direction is the actual text needing explanation, so its wording should be preserved rather than guessed.

2. verbatim

Turn 4. Verbatim recording preserves the actual phrase so the clinician can answer the real wording instead of a changed summary.

3. clinical clarification

Turn 6. Clinical clarification provides the appropriate patient-specific explanation of the instruction rather than an assistant's improvised interpretation.

4. callback request

Turn 8. A callback request records the need for contact but does not establish that the clinician has called or answered.

5. contact verification

Turn 10. Contact verification checks the appropriate details through the approved process so the response is routed correctly without assumptions.

6. urgent concern

Turn 12. An urgent concern requires its appropriate timely response and must not be downgraded merely because the call began as a wording question.

7. patient-reported concern

Turn 14. A patient-reported concern is attributed to the caller and routed for assessment rather than labeled normal or diagnosed without support.

8. aftercare

Turn 16. Aftercare concerns the actual post-treatment directions and follow-up, which should be understood rather than merely delivered.

9. instruction amendment

Turn 18. An instruction amendment is an authorized change to the directions and must be communicated and recorded clearly through the required process.

08 / Answers and explanations / continued

10. telephone note

Turn 20. The telephone note records the actual phrase, question, contact action, and pending response without claiming a clinical answer has been given.

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F / Accuracy and scope

F1. What should happen if the caller reports an urgent concern?

C. Use the designated urgent or emergency route as appropriate.

The appropriate urgent route takes precedence over routine clarification and avoids unsupported reassurance.

Return to the exercise, page 97

F2. Which note preserves the distinction?

A. Exact phrase and question logged; clinician response pending

A factual note separates the request, action, and pending response without inventing clinical resolution.

Return to the exercise, page 97

F / Guided speaking

1. review as arranged

Assistant: The quoted phrase is review as arranged.

Turn 1. The exact phrase identifies the instruction that needs clarification.

2. appointment date

Patient: The missing information is the confirmed appointment date.

Turn 2. No appointment date is confirmed in the available record.

3. interval

Assistant: I cannot invent a review interval.

Turn 3. The assistant lacks authority to create a clinical follow-up timetable.

4. unresolved

Assistant: The question is still unresolved.

Turn 4. Routing the question does not establish that an appropriate answer has already been provided.

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08 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "An unclear instruction needs an exact clinical answer". Owen: "I understand. I do not want anyone to assume that every symptom after treatment is normal, or to guess a diagnosis from one word." Farah: [missing response] Owen: "When the clinician responds, I want to be able to repeat what I understood and ask again if the explanation is still unclear." Which response belongs in the gap?

C. I will preserve any patient-reported concern accurately and follow the practice process. I should not reassure you that a described symptom is harmless without the appropriate clinical assessment.

A Not here. This line belongs to turn 8, after Owen says: "Will the dentist actually see that wording? Last time I was told a message had been logged, but I still did not know what to do." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 16, after Owen says: "When the clinician responds, I want to be able to repeat what I understood and ask again if the explanation is still unclear." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Farah's response in turn 14. It connects Owen's preceding point with Owen's follow-up. A patient-reported concern is attributed to the caller and routed for assessment rather than labeled normal or diagnosed without support.

D Not here. This line belongs to turn 12, after Owen says: "If I report a new problem as well as the wording question, should that go into the same ordinary queue automatically?" It addresses that point, not the missing turn in the supplied exchange.

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Review 1 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Material placed to restore a prepared tooth area as clinically indicated. Which term fits?

D. filling

A Not here. restorative dentistry: Dental care concerned with repairing or restoring teeth and function. The usual combination is "explain restorative dentistry".

B Not here. treatment plan: The proposed sequence of dental care discussed with the patient. The usual combination is "explain the treatment plan".

C Not here. booking discrepancy: A difference between the recorded booking and the expected visit. The usual combination is "resolve a booking discrepancy".

D Correct. filling: Material placed to restore a prepared tooth area as clinically indicated. The usual combination is "schedule a filling".

Return to the exercise, page 26

2. A colleague needs the term for this meaning: Documentation of a particular processing cycle. Which term fits?

C. cycle record

A Not here. pack integrity: The condition of the packaging and its protective barrier. The usual combination is "check pack integrity".

B Not here. instrument pack: A packaged set of dental instruments with a defined processing and use status. The usual combination is "identify the instrument pack".

C Correct. cycle record: Documentation of a particular processing cycle. The usual combination is "trace the cycle record".

D Not here. traceability: Ability to connect an item with relevant identity and processing history. The usual combination is "maintain traceability".

Return to the exercise, page 26

Review 1 / Explanations / continued

3. Rebuild the exchange from "A consultation is not a confirmed filling". Lena: "I would also like to understand the cost before agreeing. I received a number over the phone, but I do not know exactly what it covered." Amir: [missing response] Lena: "The receptionist also mentioned my insurer. Should I assume that the amount above that estimate will be paid by insurance?" Which response belongs in the gap?

B. We should clarify the treatment estimate and which service it described. A consultation estimate and an estimate for a particular restoration may not cover the same work.

A Not here. This line belongs to turn 16, after Lena says: "Thank you. I want the dentist to know I expected treatment, but I do not want anyone to think I am demanding a procedure that might not be right." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Amir's response in turn 10. It connects Lena's preceding point with Lena's follow-up. The treatment estimate must specify the proposed care and cost basis rather than remain an unexplained number.

C Not here. This line belongs to turn 12, after Lena says: "The receptionist also mentioned my insurer. Should I assume that the amount above that estimate will be paid by insurance?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 20, after Lena says: "All right, I will speak to the dentist first. Please mention the time off work too. Another appointment would take some arranging." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 26

Review 1 / Explanations / continued

4. Rebuild the exchange from "The pack is present; readiness is not verified". Tessa: "If another pack is confirmed, should I describe the entire issue as closed, or distinguish the replacement from the original record question?" Bruno: [missing response] Tessa: "Understood. I will record the actual pack status, whom I contacted, and any verified replacement details, without claiming a failure that has not been found." Which response belongs in the gap?

A. Distinguish them. The cycle record issue may require further review even after the immediate setup has an acceptable alternative. Do not erase the first concern from the history.

A Correct. This is Bruno's response in turn 18. It connects Tessa's preceding point with Tessa's follow-up. The cycle record concerns the original processing history, which still needs the appropriate review even if a replacement is available.

B Not here. This line belongs to turn 8, after Tessa says: "Would a replacement solve the immediate setup problem? I heard there might be another pack, but I have not checked its identity or status." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 12, after Tessa says: "I can give you the identifiers shown without altering anything. I do not want to copy a number from another pack into the missing record." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 4, after Tessa says: "It has not been used. I have not released it, and I do not have authority to waive the missing check just because the room is busy." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 26

Review 2 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: A tray used to organize the relevant dental instruments. Which term fits?

D. instrument tray

A Not here. distinguishing detail: The feature that separates one possible request from another. The usual combination is "request the distinguishing detail".

B Not here. explorer: A dental examination instrument used by appropriately trained personnel. The usual combination is "identify the explorer".

C Not here. handpiece: A powered dental instrument used with specified attachments. The usual combination is "clarify the handpiece type".

D Correct. instrument tray: A tray used to organize the relevant dental instruments. The usual combination is "confirm the instrument tray".

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2. A colleague needs the term for this meaning: Cone-beam computed tomography; three-dimensional X-ray imaging used for appropriate indications. Which term fits?

B. CBCT

A Not here. diagnostic significance: The relevance of information to identifying a condition. The usual combination is "ask about diagnostic significance".

B Correct. CBCT: Cone-beam computed tomography; three-dimensional X-ray imaging used for appropriate indications. The usual combination is "identify the CBCT study".

C Not here. artifact: An image feature introduced by a technical or other non-anatomical factor. The usual combination is "report a possible artifact".

D Not here. radiograph: An image produced using X-rays. The usual combination is "identify the radiograph".

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Review 2 / Explanations / continued

3. Rebuild the exchange from "The pack is present; readiness is not verified". Tessa: "If we locate the original record, we also need to know that it belongs to P18 rather than a similar pack from another processing load." Bruno: [missing response] Tessa: "I can give you the identifiers shown without altering anything. I do not want to copy a number from another pack into the missing record." Which response belongs in the gap?

C. Correct. Traceability connects the item to the relevant history. A complete-looking record for a different load would not answer the question about this pack.

A Not here. This line belongs to turn 12, after Tessa says: "I can give you the identifiers shown without altering anything. I do not want to copy a number from another pack into the missing record." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 16, after Tessa says: "I will tell the dentist that I have raised the specific gap and that you are checking the record and a possible verified alternative." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Bruno's response in turn 10. It connects Tessa's preceding point with Tessa's follow-up. Traceability connects the particular pack to its relevant processing history rather than accepting an unrelated record.

D Not here. This line belongs to turn 2, after Tessa says: "Bruno, please pause the setup. Pack P18 is present, but I cannot confirm its required readiness documentation, and the dentist is about to begin." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 49

Review 2 / Explanations / continued

4. Rebuild the exchange from "Explain the route to an answer, not the image". Marco: "Would an earlier image matter? Has the dentist compared this with my previous image?" Suki: [missing response] Marco: "Please ask what the patch means and whether it matches the examination. That is what I need to understand first." Which response belongs in the gap?

A. You can ask whether a comparison image was relevant and reviewed. I will not say that comparison has happened unless it is confirmed in the actual explanation.

A Correct. This is Suki's response in turn 18. It connects Marco's preceding point with Marco's follow-up. A comparison image is another image used for a defined clinical comparison; the scenario does not establish that one was reviewed.

B Not here. This line belongs to turn 8, after Marco says: "Then I should ask what the feature actually represents. I would also like to know whether anything else in the examination supports the same conclusion." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 16, after Marco says: "If a treatment is recommended, I want to understand why that treatment follows from the evidence rather than think the picture makes the decision automatically." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 4, after Marco says: "I thought you could explain it because you displayed it. Who should I ask about that darker patch?" It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 49

Review 3 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: An appropriate action intended to support comfort during care. Which term fits?

C. comfort measure

A Not here. previous experience: An earlier event affecting expectations or concerns. The usual combination is "acknowledge a previous experience".

B Not here. communication preference: The person's preferred way of receiving or giving information. The usual combination is "clarify a communication preference".

C Correct. comfort measure: An appropriate action intended to support comfort during care. The usual combination is "discuss comfort measures".

D Not here. local anesthesia: A clinical method of reducing sensation in a defined area. The usual combination is "discuss local anesthesia".

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2. A colleague needs the term for this meaning: A patient's clinical record. Which term fits?

B. chart

A Not here. tooth notation: A system for identifying teeth in records. The usual combination is "confirm the tooth notation system".

B Correct. chart: A patient's clinical record. The usual combination is "verify the chart".

C Not here. patient identifier: Information used to distinguish a particular patient under the approved process. The usual combination is "verify patient identifiers".

D Not here. traceable correction: An amendment that preserves who changed what and when. The usual combination is "make a traceable correction".

Return to the exercise, page 73

Review 3 / Explanations / continued

3. Rebuild the exchange from "Explain the route to an answer, not the image". Marco: "There is also a mark on the screen. I cannot tell whether it is part of the original image or something someone added to point out an area." Suki: [missing response] Marco: "Could it be a technical issue instead? I am not asking you to decide that, but I would like the dentist to address the possibility." Which response belongs in the gap?

A. The clinician can clarify that image annotation and what it refers to. I should not guess the purpose of a mark or present it as proof of a particular condition.

A Correct. This is Suki's response in turn 10. It connects Marco's preceding point with Marco's follow-up. An image annotation is an added label or mark whose purpose and source need explanation rather than assumed diagnostic meaning.

B Not here. This line belongs to turn 20, after Marco says: "Please ask what the patch means and whether it matches the examination. That is what I need to understand first." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 14, after Marco says: "I appreciate that. I do not want reassurance if it is not justified, but I also do not want a frightening answer based on a guess." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 8, after Marco says: "Then I should ask what the feature actually represents. I would also like to know whether anything else in the examination supports the same conclusion." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 73

Review 3 / Explanations / continued

4. Rebuild the exchange from "A shared surname is not a verified match". Diego: "Can we check the details in the staff area? Both patients' names are visible here, and I do not want this conversation carrying into the waiting room." Priya: [missing response] Diego: "Until the check is complete, my update will say that the match is unverified, the folder has not been used, and you are coordinating the review." Which response belongs in the gap?

B. That keeps this a confidential handoff. We need enough information to resolve the issue, without exposing either patient's details to people who do not need them.

A Not here. This line belongs to turn 4, after Diego says: "There are two patients named Morgan. I have not used the folder or disclosed its contents, and I cannot establish which identifier belongs with this visit." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Priya's response in turn 18. It connects Diego's preceding point with Diego's follow-up. A confidential handoff shares the relevant information through an appropriately protected route instead of exposing it to unrelated people.

C Not here. This line belongs to turn 2, after Diego says: "Priya, I have an identifier mismatch before using this folder. The appointment list ends four-six-two, but the folder label ends four-two-six." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 16, after Diego says: "And if the review reveals two records for one person, that would still be a separate authorized process rather than something I do because the surname matches." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 73

Review 4 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Authorized clinical instructions for the laboratory work. Which term fits?

B. laboratory prescription

A Not here. shade: The specified color characteristics of a restoration or appliance. The usual combination is "confirm the shade reference".

B Correct. laboratory prescription: Authorized clinical instructions for the laboratory work. The usual combination is "verify the laboratory prescription".

C Not here. fit assessment: Clinical assessment of whether the item fits as required. The usual combination is "obtain a fit assessment".

D Not here. estimated delivery: A forecast of when the item may arrive. The usual combination is "qualify estimated delivery".

Return to the exercise, page 99

2. A colleague needs the term for this meaning: A request for a return call, not confirmation that the call occurred. Which term fits?

B. callback request

A Not here. postoperative: Relating to the period after an operation or procedure. The usual combination is "clarify postoperative instructions".

B Correct. callback request: A request for a return call, not confirmation that the call occurred. The usual combination is "log a callback request".

C Not here. emergency response: The immediate appropriate response to an emergency under local arrangements. The usual combination is "use the emergency response".

D Not here. swelling: Enlargement of tissue whose meaning depends on clinical context. The usual combination is "report swelling accurately".

Return to the exercise, page 99

Review 4 / Explanations / continued

3. Rebuild the exchange from "A shared surname is not a verified match". Diego: "Should I ask the person a leading question with one of the numbers, or follow the usual identification questions without suggesting the answer?" Priya: [missing response] Diego: "The folder also contains a dental chart, but I have not used its contents to guess whether it looks like the patient I remember." Which response belongs in the gap?

D. Follow our approved patient identifier checks. These partial numbers alone are not enough, and we should not improvise a shortcut that makes an uncertain match appear confirmed.

A Not here. This line belongs to turn 16, after Diego says: "And if the review reveals two records for one person, that would still be a separate authorized process rather than something I do because the surname matches." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 18, after Diego says: "Can we check the details in the staff area? Both patients' names are visible here, and I do not want this conversation carrying into the waiting room." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Diego says: "Priya, I have an identifier mismatch before using this folder. The appointment list ends four-six-two, but the folder label ends four-two-six." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Priya's response in turn 10. It connects Diego's preceding point with Diego's follow-up. Patient identifiers must be verified through the actual approved process, not by treating a prompted guess as reliable confirmation.

Return to the exercise, page 99

Review 4 / Explanations / continued

4. Rebuild the exchange from "An unclear instruction needs an exact clinical answer". Owen: "And if the clinician changes the direction, I would like the new wording clear. I do not want two different instructions without knowing which applies." Farah: [missing response] Owen: "Please record the phrase and my question about its meaning. You have not given me a new clinical direction." Which response belongs in the gap?

A. Any instruction amendment needs to be communicated and recorded through the proper process. I will not create a competing instruction myself or silently replace the original text.

A Correct. This is Farah's response in turn 18. It connects Owen's preceding point with Owen's follow-up. An instruction amendment is an authorized change to the directions and must be communicated and recorded clearly through the required process.

B Not here. This line belongs to turn 6, after Owen says: "I am glad you said that. I do not need a general answer copied from someone else's visit; I need to know what the direction means for mine." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Owen says: "Farah, I am home with the instruction sheet, but one sentence has me stuck. Could someone explain it before I guess what I am supposed to do?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 8, after Owen says: "Will the dentist actually see that wording? Last time I was told a message had been logged, but I still did not know what to do." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 99

Additional case 1 / Explanations

Extended conversation

1. Universal

Turn 2. Universal number twelve identifies the permanent upper-left first premolar; the identical digits in FDI notation identify a different tooth.

2. FDI

Turn 4. FDI two-four corresponds to the intended permanent upper-left first premolar; FDI one-two is not the same tooth as Universal twelve.

3. notation system

Turn 6. The notation system must accompany the tooth designation so a reader knows how to interpret the digits.

4. distal

Turn 8. The confirmed MOD entry includes distal as well as mesial and occlusal; recording MO would omit one of the specified surfaces.

5. planned

Turn 10. The dentist explicitly confirms planned status; the dialogue supplies no completed procedure to document or bill as completed.

6. audit trail

Turn 14. An audit trail preserves the source wording and the dated, attributable clarification instead of concealing the original ambiguity.

Return to the conversation, page 102

Transfer exchange

1. 12

Assistant: In FDI notation, the confirmed tooth is 12 .

Turn 1. FDI twelve identifies the permanent upper-right lateral incisor specified by the dentist.

Additional case 1 / Explanations / continued

2. 7

Dentist: Its Universal designation is 7 .

Turn 2. Universal seven identifies that same permanent upper-right lateral incisor, not Universal twelve.

3. existing

Assistant: The confirmed record status is existing .

Turn 3. The supplied instruction describes an existing restoration rather than work newly completed at this visit.

4. audit trail

Dentist: Preserve the source and clarification in the audit trail .

Turn 4. The correction must remain attributable and traceable rather than silently erasing the imported notation.

Return to the conversation, page 105

Additional case 2 / Explanations

Extended conversation

1. no known drug allergies

Turn 2. The existing no-known-allergies entry conflicts with a newly reported possible drug reaction and needs review; it should not be carried forward unquestioned.

2. drug name

Turn 4. The drug name is unknown in the supplied account; choosing a familiar antibiotic would invent information rather than clarify the history.

3. reported reaction

Turn 6. The reported reaction is Ravi's account of red, itchy patches, not a confirmed allergy diagnosis or a clinician's documented conclusion.

4. unknown

Turn 8. Unknown timing is a missing fact; it must not be converted into an immediate reaction or a statement that no reaction occurred.

5. information source

Turn 10. The information source is the patient's recollection until another record is actually obtained and checked; a possible pharmacy record is not yet corroborating evidence.

6. clinical review

Turn 17. Clinical review is needed for assessment and advice; collecting the history does not authorize the assistant to change medicines or clear the patient for treatment.

Return to the conversation, page 107

Transfer exchange

1. nausea

Assistant: The symptom the patient recalls is nausea .

Turn 1. Nausea is the supplied symptom; replacing it with rash would change the reported history.

Additional case 2 / Explanations / continued

2. unknown

Patient: The medicine's name is unknown .

Turn 2. The patient cannot identify the medicine, so the assistant must not invent a drug name.

3. allergy

Assistant: This is a reported reaction, not a confirmed allergy .

Turn 3. The facts provide an account requiring assessment, not a clinical conclusion that the reaction was allergic.

4. review

Patient: The advice and history update still need clinician review .

Turn 4. The report has been routed but not clinically assessed or resolved in the supplied exchange.

Return to the conversation, page 109

Additional case 3 / Explanations

Extended conversation

1. usable stock

Turn 2. Only the six verified, in-date syringes count as usable stock; the four expired syringes remain physically present but unavailable for use.

2. lot number

Turn 4. The lot number distinguishes C27 from expired C18 even though their product and shade are the same.

3. six

Turn 6. Twelve target syringes minus six usable syringes leaves a six-syringe replenishment requirement, with no outstanding order to subtract.

4. first-expiry-first-out

Turn 8. First-expiry-first-out prioritizes the earliest expiry among eligible stock; it does not authorize using already expired material.

5. expiry date

Turn 10. The expiry date is a separate label field from receipt date and lot number; opening or moving a pack does not extend it.

6. instructions for use

Turn 14. The instructions for use are product-specific; another material's storage, handling, or curing directions cannot be applied merely because the products are used together.

Return to the conversation, page 111

Transfer exchange

1. 8

Assistant: Physical quantity is 8 syringes.

Turn 1. Five usable plus three expired units equals eight physically present, but not eight usable.

Additional case 3 / Explanations / continued

2. 10

Lead: The replenishment requirement is 10 syringes.

Turn 2. The target of fifteen less five usable syringes requires ten additional syringes.

3. 2

Assistant: That quantity equals 2 supplier cartons.

Turn 3. Ten required syringes divided by five per carton equals two cartons.

4. received

Lead: New stock has not yet been received .

Turn 4. Preparing a purchase request does not establish approval, shipment, or physical receipt of supplies.

Return to the conversation, page 114

Find a phrase by purpose

Clarifying the appointment purpose

Acknowledge the expectation: I understand you expected a filling today.

State the record: The schedule shows a thirty-minute consultation.

Name the difference: That differs from the treatment you were expecting.

Full phrase bank, page 7

Room readiness and safe clarification

Interrupt clearly: Please pause; I cannot confirm the required readiness check for P18.

Identify the item: The concern is about pack P18 in this room.

State the gap: The required documentation is not available for verification.

Full phrase bank, page 19

Chairside requests and read-backs

Signal the gap: I heard tray, but I missed the word before it.

Name the interruption: Another conversation covered the rest of the request.

Offer the alternatives: Did you mean the instrument tray or the impression tray?

Full phrase bank, page 31

Imaging questions and authorization

Acknowledge the question: You want to know whether the image means this tooth needs treatment.

State the role: I am not authorized to interpret the image.

Offer the next step: I will ask the dentist to explain the findings with you.

Full phrase bank, page 41

Find a phrase by purpose / continued

Patient comfort, questions, and pauses

Acknowledge anxiety: Thank you for telling me that you are worried.

Invite the specific concern: What would you like the dentist to know before starting?

Avoid a guarantee: I cannot promise that you will feel nothing.

Full phrase bank, page 54

Records, identifiers, and factual handoffs

State the discrepancy: The list and folder show different identifier endings.

Read the values clearly: The list ends four-six-two; the label ends four-two-six.

Name the shared name: Two patients in the records share this surname.

Full phrase bank, page 65

Laboratory cases and delivery expectations

Identify the case: I am checking case C62 for Tuesday's appointment.

Request the stage: What is the current production and dispatch status?

Repeat the estimate: You estimate arrival Tuesday by noon, without a guarantee.

Full phrase bank, page 78

Aftercare questions and follow-up calls

Welcome the question: Thank you for calling to check the instruction.

Ask for exact wording: Could you read the phrase exactly as it appears?

Preserve the source: I will record that as wording from your instruction sheet.

Full phrase bank, page 90

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References and publication notes

All cases, organizations, figures, and conversations are fictional teaching material, not transcripts. Communication practice does not replace qualified advice or current workplace procedures.

[1] American Dental Association. Dental Assistant.

United States career and role terminology. Permitted duties vary by jurisdiction, qualifications, and actual authorization; examples do not grant clinical authority. Source checked 10 October 2026.

https://www.ada.org/resources/careers/career-pathways/dental-assistant

[2] CDC. Sterilization and Disinfection in Dental Settings.

Background on processing terminology and the importance of verification records. The fictional readiness dialogue is not an instrument-reprocessing procedure. Source checked 10 October 2026.

https://www.cdc.gov/dental-infection-control/hcp/summary/sterilization-disinfection.html

[3] General Dental Council. Principle Three: Obtain Valid Consent.

UK professional context for ongoing patient communication and consent. Original language scenarios do not substitute for the applicable consent process. Source checked 10 October 2026.

https://standards.gdc-uk.org/pages/principle3/principle3

[4] General Dental Council. Principle Four: Maintain and Protect Patients Information.

UK professional context for clear, accurate, confidential records and traceable amendments. Follow actual local requirements for records and disclosure. Source checked 10 October 2026.

https://standards.gdc-uk.org/pages/principle4/principle4

Companion course

The website offers the eight-lesson course and related practice. This book can be used independently. No audio access or external account is needed for its exercises.

https://englishladder.com/efsp-dental-assistants.html

Clarify the request. Support the patient.

Clarify appointments and chairside requests, support patient questions, check records, track laboratory work, and communicate aftercare concerns. Three further cases cover tooth notation, medical history, and dental-material stock.

Clarifying the appointment purpose

A thirty-minute booking describes allocated time, not clinical suitability or consent for a filling. Explain the visit type and arrange the clinical discussion. Do not convert a hoped-for treatment into a confirmed procedure because the patient has arrived.

Room readiness and safe clarification

Present, packaged, clean, disinfected, and sterile are not interchangeable statuses. Recognition of an indicator or a sealed pack does not replace the required combined checks, records, and manufacturer directions. This unit practices clarification, not a processing protocol.

Chairside requests and read-backs

Instrument tray and impression tray share a word but identify different items. Repeat the distinguishing term, not only tray. The same principle applies to size, identifier, side, and timing when those details are part of the actual request.

Imaging questions and authorization

Acquiring, labeling, displaying, and interpreting an image are different activities. The actual role and local rules determine who may do each. A visible feature or a technical term does not establish a diagnosis or make an assistant authorized to provide one.

Phil Stilwell

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